MSC MARRIAGE AND FAMILY THERAPY

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MFT 702

Theories of Family Therapy

14 teaching weeks · 11 practicals · 19 worked cases · 6 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

> “Nine major schools, each treated as an answer to a specific question.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 702 — Theories of Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 702	**Theories of Family Therapy**	One	1.71

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Course leader's summary. Nine major schools, each treated as an answer to a specific question. You will learn each model well enough to do a first session in it, critique it, and integrate it — and you will practise moving between models on the same family until the choice becomes deliberate rather than habitual.

How to use this book

Theory is learned in three passes. **Pass one:** understand the model's core question and its intervention. **Pass two:** apply it to a case you know. **Pass three:** critique it and ask what it cannot see. Each week in this book gives you all three, plus the practical that makes the pass real. Do not memorise lists of techniques; memorise what problem each model believes it is solving.

1. Course description

There is no single "correct" family therapy. There are traditions, each with a theory of how problems develop, a theory of change, a preferred focus of attention, and a characteristic set of techniques. A clinician who knows only one model does not have a theory; they have a habit.

This course therefore pursues three aims: **comparative competence** (you can describe and demonstrate each major model), **critical appraisal** (you can name each model's evidence base and its blind spots), and **integration** (you can combine two models coherently in one case rather than dithering between them).

Organising question. For each model, ask: What does this model believe is the problem, and what does it therefore do first?

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the theoretical foundations, core concepts and change mechanisms of nine major models	Exam
LO2	Demonstrate a model-consistent first session for at least three models	Skills assessment
LO3	Compare models on focus, technique, role of the therapist and evidence	Comparative matrix
LO4	Identify the blind spots and cultural assumptions of each model	Essay
LO5	Construct a coherent integrative plan for a single case using two models	Case assignment
LO6	Justify model choice with reference to the client, the evidence and your own competence	Viva

3. Assessment

Component	Weight	Due
Comparative matrix (nine models × six dimensions)	20%	Week 7
Observed model-consistent session (20 min)	20%	Weeks 8–10
Integrative case study (2,500 words)	30%	Week 12
Written examination	30%	Week 15

4. The fourteen-week plan

Week	Model / topic	Practical	Key text
1	How to compare theories: six dimensions	Matrix construction	Nichols ch. 1
2	Psychoanalytic and object-relations family therapy	Genogram + interpretation exercise	Scharff & Scharff
3	Bowen's multigenerational theory	Detriangling role-play	Bowen (1978)
4	Structural family therapy	Enactment and boundary-making	Minuchin (1974)
5	Strategic and MRI brief therapy	Directive design	Haley & Madanes
6	Milan systemic therapy	Circular questioning	Palazzoli et al. (1980)
7	Experiential and human validation (Satir, Whitaker)	Sculpture exercise	Satir (1988)
8	Cognitive-behavioural and behavioural couple therapy	Behavioural exchange design	Epstein & Baucom
9	Emotionally focused therapy	EFT enactment	Johnson (2019)
10	Solution-focused brief therapy	Miracle question and scaling	de Shazer (1985)
11	Narrative therapy	Externalising conversation	White (2007)
12	Integration: combine two models without incoherence	Case integration workshop	Lebow (2014)
13	Evidence, culture and critique of each model	Debate	Sprenkle et al.
14	Your theoretical position	Position paper presentation	—

Week 1 — How to compare theories: six dimensions

Every model in this course will be described on the same six axes. Learn to fill the grid from memory and you will be able to enter any family therapy conversation — an interview, a case conference, a viva — with something more useful than an opinion. Learn the models as nine separate stories and you will have nine stories and no method.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The problem with model surveys	Two clinicians, one family, incompatible first questions	Predict what each will do in the first five minutes
10–25	The six dimensions	Define each, with a counter-example	Fill two rows of the matrix from memory
25–40	Theory of problem as master dimension	Show how the other five follow from it	Derive a model's techniques from its theory of problem
40–55	Three levels of knowing a model	Describe, demonstrate, critique	Rate themselves on each level for one model
55–70	Comparing three models on one case	Fill the grid live from the Adeyemi material	Add a fourth model
70–85	The reduction test and the charity test	Apply both to a claim about a model	Test two claims in pairs
85–90	Set the assessment	The matrix, its due date and its marking	Plan their matrix file

1.2 Why comparison is a skill

A clinician who knows one model does not have a theory; they have a habit. That sentence is the premise of the course, and this week explains why it is true and what to do about it.

What comparison is for. Four uses, each practical.

First, it makes the first session deliberate. The first five minutes of a family session are shaped by a model whether or not the therapist has chosen one. The structural therapist moves to the hierarchy; the narrative therapist asks what name the family would give the problem; the emotionally focused therapist watches for the cycle. Comparison allows the therapist to know which of these they are doing, and to choose.

Second, it protects against the single-story error. Every model has a characteristic blind spot, and the blind spot is usually the same as its strength. A therapist who cannot name the blind spot of their preferred model cannot see the family that the model is failing.

Third, it makes referral and supervision possible. Knowing that a family needs something you do not do is the beginning of professional judgement. Knowing it in the vocabulary of the field — "this family needs a structural first phase and then narrative work" — makes the referral legible to a colleague.

Fourth, it is examinable, and it is examined everywhere. The comparative matrix, the viva, the case study: all three assessments in this course are applications of the six dimensions. Students who build the matrix properly in Week 1 are doing the exam in October.

What comparison is not. It is not a listing of techniques, a history of ideas, or an argument about which model is best. It is a method for answering a specific question about any model: what does this model believe the problem is, and what does it therefore do first?

1.2a How to study nine models in fourteen weeks

The course is short and the field is large, so the method matters as much as the effort. Five practices distinguish students who can use the models from students who can list them.

Practice 1 — one primary text per model, chosen for size. Not the complete works; a single book or long chapter that the model's author wrote for practitioners. For each model in this course, the reading list gives one such text. Read it in the week, at speed, for the model's own vocabulary; the secondary summaries will then have something to attach to.

Practice 2 — the one-sentence discipline. At the end of each reading session, write the theory of problem in one sentence, in the author's terms, and then write a second sentence saying what the author believes is not the problem. The second sentence is what distinguishes models, because every model defines itself against a predecessor.

Practice 3 — apply the model to a family you know. Not a hypothetical family: the Adeyemi case, or a family from placement, or the running case in the seminars. Each week, write one page applying the new model to the same family. By Week 11 you will have eleven pages on one family, which is the best comparative document you will ever own, because the family is held constant and only the lens changes.

Practice 4 — record yourself at level 2 once per model. Twenty minutes with a role-played family, in the model's terms, recorded. Watch it with the model's signature techniques written in front of you and count how often you used them. Most students find that they used the techniques of the model they learned first, whatever model they were attempting, which is the clearest possible demonstration of why deliberate practice exists.

Practice 5 — keep a disagreement file. Every time a model's claim conflicts with another's, or with the evidence, or with your own experience, write the conflict down in two sentences with the date. Bring the file to the seminars. The disagreements are the material of the essay, and they are what the viva will test.

What the practices have in common. They all convert reading into something that can be observed: a sentence, a page, a recording, a note. Reading alone produces the illusion of knowledge that the first seminar exposes, and the practices are designed to expose it earlier, in private, where it costs nothing.

1.3 The six dimensions

The grid is the spine of the course. Each dimension is a question with a determinate answer for each model, and the answers are not independent: the first dimension generates most of the others.

Dimension	The question it answers
Theory of problem	How does this model believe problems develop and persist?
Unit of focus	Individual, dyad, family structure, meaning system, or context?
Theory of change	What must happen for things to improve?

Dimension	The question it answers
Therapist position	Director, coach, collaborator, witness, expert?
Signature techniques	What does this model characteristically do?
Evidence base	What is the strength and quality of supporting research?

Dimension 1 — theory of problem. How does this model believe problems develop and persist? This is the master dimension. A model that believes problems are maintained by structure will intervene in structure; a model that believes they are maintained by meaning will intervene in meaning; a model that believes they are maintained by unexpressed emotion will intervene in emotion. Everything else follows, and a student who cannot state the theory of problem in one sentence has not learned the model.

Dimension 2 — unit of focus. What is the model looking at? The individual in context, the dyad, the triadic or hierarchical structure, the family's meaning system, the multigenerational transmission, or the wider context of institutions and culture. The unit determines what counts as data and what counts as noise. To the structural therapist, a mother's tearfulness is data about the family's organisation; to the emotion-focused therapist, the organisation is data about the tearfulness.

Dimension 3 — theory of change. What must happen for things to improve, and in what order? Some models believe insight precedes change; some that change precedes insight; some that a small difference is sufficient; some that the system must be destabilised before it can reorganise. This dimension predicts the shape of the therapy — its length, its pacing, and what the therapist will do when nothing is happening.

Dimension 4 — therapist position. Director, coach, collaborator, witness, expert, non-expert, consultant. The position determines who speaks, who decides, and what the family is told about what is happening. It is the dimension most visible to families, and the one trainees most often adopt by accident.

Dimension 5 — signature techniques. What does this model characteristically do? Enactment, reframing, circular questioning, genogram, sculpture, externalising, miracle question, behavioural exchange, two-chair work. Techniques are the easiest dimension to learn and the least useful on their own, because a technique divorced from its theory of problem is a gesture.

Dimension 6 — evidence base. What is the strength and quality of the research support, for what presentations, and with what limits? This dimension is the one most often omitted in textbooks and most likely to be tested in a viva, because a clinician's claim about evidence is a claim about what they owe a family.

Two further dimensions worth adding to the matrix. The six are the required grid; two more make it a professional instrument.

Dimension 7 — cultural assumptions. Every model carries a picture of a healthy family, and that picture has a culture. A model that prizes individuation will read a three-generational household differently from a model that prizes interdependence. The

question is not whether the model has cultural assumptions — all do — but what they are, and what they make the model unable to see.

Dimension 8 — the characteristic blind spot. What does this model systematically not notice? The blind spot is not a defect to be apologised for; it is the shadow of the model's strength, and it is the most useful thing a comparative therapist knows.

1.4 Theory of problem as the master dimension

The claim that the first dimension generates the others is testable in an exercise that takes ten minutes and is worth a term of reading.

The exercise. Write down, in one sentence, what a model believes maintains the problem. Then derive, without looking anything up, what its unit of focus must be, what its theory of change must be, what position its therapist must occupy, and two techniques it must use.

Worked derivation 1 — structural. Theory of problem: difficulties are maintained by the family's organisation — unclear hierarchies, inappropriate boundaries, cross-generational coalitions. Unit of focus: the whole family as a structure, with subsystems. Theory of change: the structure must change in the room, through action, before understanding follows. Therapist position: active director, at times stage-manager. Techniques: enactment, unbalancing, boundary-making, spatial rearrangements.

Worked derivation 2 — narrative. Theory of problem: difficulties are maintained by the dominant stories that families and institutions hold about their members, and by the practices those stories make possible. Unit of focus: the meaning system, and the social context in which meanings circulate. Theory of change: the story must be made visible, the person separated from the problem, and a preferred account thickened. Therapist position: collaborative, curious, decentred but influential. Techniques: externalising, re-authoring, unique outcomes, definitional ceremonies.

Worked derivation 3 — emotion-focused. Theory of problem: distress is maintained by rigid negative interactional cycles that organise around unexpressed primary emotion and unmet attachment needs. Unit of focus: the couple's cycle, and the attachment bond that the cycle protects. Theory of change: the cycle must be slowed, the primary emotion accessed and expressed, and a new cycle enacted in the room. Therapist position: active, empathic, a process consultant who stages the encounter. Techniques: tracking the cycle, reframing in terms of the cycle, the enactment, and the "hold me tight" conversation.

What the derivations demonstrate. That a model is a system, not a list. Students who can generate the five other dimensions from the first have understood a model; students who have memorised six columns have not. The examination will ask for a derivation, in some form, every year.

1.4a The six dimensions as questions you can ask a family

The grid is usually taught as an instrument for reading textbooks. It is more useful as an instrument for the room, because each dimension can be converted into a question that changes what the family tells you.

From theory of problem to a question about maintenance. Model reading: the model believes something specific maintains the problem. Question: "What keeps this going, do you think? Not how it started — what keeps it going now?" Families answer this question with a mechanism, and the mechanism they describe is often more accurate than the formulation in the referral.

From unit of focus to a question about who matters. Question: "Who else is affected by this — and who else has an opinion about it?" In families where the meaningful unit is wider than the household, this question recruits the grandmother, the aunt or the pastor into the conversation before the therapist has decided they are relevant.

From theory of change to a question about what has helped before. Question: "When it has been better, even for a week, what made the difference?" This question tests the model's theory of change against the family's own experience: if the family's answer is "we agreed one thing and stuck to it", a structural first move is indicated; if it is "we stopped talking about it and it faded", a solution-focused approach has better odds.

From therapist position to a question about what the family wants from you. Question: "Some families want me to tell them what to do; others want me to help them work it out. Which would be more useful to you?" The answer is not binding, and asking it prevents the commonest early rupture: a family expecting direction receiving exploration, or the reverse.

From signature technique to a question about what the family has already tried. Question: "What have you tried, and what happened?" This is the shortest route to the model's technique, because the family's failed solutions are usually the inverse of what the model would do.

From evidence base to a question about the family's own data. Question: "Would you be willing to rate how things are each week, so we can see whether this is working rather than guessing?" The evidence dimension made operational.

The point of the exercise. A dimension that cannot generate a question is a dimension the student has learned verbally. The test of understanding a model is not that you can describe its unit of focus but that you know what to ask a family in order to work with it.

1.5 Three levels of knowing a model

A student can be at three different levels with the same model, and the levels are assessed separately.

Level 1 — describe. You can state the theory of problem, the unit of focus, the theory of change, the therapist position, two or three techniques and the evidence base, in your own words, without notes. This is the level required for the written examination, and it is the level at which most textbooks stop.

Level 2 — demonstrate. You can conduct twenty minutes of a model-consistent session with a role-played family, so that an observer who knows the model can identify it from the transcript without being told. This is the level required for the observed session, and it

is much harder than describing, because it requires knowing what the model sounds like: the actual sentences, the pacing, the shape of the first ten minutes.

Level 3 — critique. You can state the model's blind spot, its cultural assumptions, the presentations for which the evidence is strong and weak, and what you would do when the model fails. This is the level required for the essay and the viva, and it is the level at which a clinician becomes safe to practise, because it is the level at which they know when to stop.

The diagnostic. For each model in this course, rate yourself 0–3 on each level: 0 (cannot), 1 (with notes), 2 (reliably), 3 (can teach it). Bring the grid to the Week 7 practical. Most students overrate level 1 and underrate level 2, and the pattern is worth knowing about oneself.

Why the levels matter for the matrix. The comparative matrix is a level 1 instrument. It is necessary and it is not sufficient. A student who can fill the matrix and cannot demonstrate a single model has learned a taxonomy, not a therapy.

1.6 Comparing three models on one family

The Adeyemi family, from MFT 701, is the running case. The grid below is filled for three models, and the fourth column is the week's exercise.

Dimension	Structural	Bowen	Solution-focused
Theory of problem	Maintained by unclear hierarchy, diffuse or rigid boundaries, cross-generational coalitions	Maintained by anxiety transmitted across generations and absorbed by the most vulnerable member; triangles	Maintained by a problem-saturated description and by the family's attention to what is not working
Unit of focus	Whole family as a structure, with subsystems and boundaries	Multigenerational emotional system; triangles; differentiation	The family's description of the problem and its preferred future
Theory of change	Restructure in the room; action before insight	Lower reactivity, raise differentiation, detriangle	Build a description of the solution and amplify existing exceptions
Therapist position	Active director, stage-manager	Coach, neutral observer of process	Curious collaborator, expert in process not in lives
Signature techniques	Enactment, unbalancing, boundary-making	Genogram, coaching, detriangling questions	Miracle question, scaling, exception-seeking
Evidence base	Moderate: strong for adolescent behaviour problems and eating disorders in adapted forms	Thin: influential, widely taught, little trial evidence	Moderate for depression and behaviour problems in brief formats; strong evidence for the components (goal setting, scaling)
Cultural assumption	A family should have a clear hierarchy with parents in charge	Differentiation is a good; the individual should think for themselves	The family can describe a preferred future and act on it

Dimension	Structural	Bowen	Solution-focused
Blind spot	Can impose a structure the family experiences as foreign; may miss meaning, loss and material conditions	Slow; may intellectualise; may under-attend to practical and institutional pressure	May bypass the function of the symptom and the history that sustains it

What the grid shows at a glance. Three competent clinicians, one family, three different first sessions — and none of them is wrong. The grid also shows why model loyalty is a temperamental matter: the structural column appeals to therapists who want to act, the Bowen column to therapists who want to understand, the solution-focused column to therapists who want to build. The professional question is not which temperament is best but which the family needs.

The fourth column. Fill it for narrative therapy before the seminar, from the reading, and bring it. The exercise is deliberately placed before the seminar so that students discover what they cannot yet do.

1.6a The matrix as a clinical instrument: choosing under pressure

In a real clinic the matrix is used in ninety seconds, in a corridor, with a referral in your hand. Three ways it earns its keep.

Case 1 — the referral that does not fit your model. A family is referred for a child's behaviour problems. You work structurally. The referral letter says the parents separated two years ago, the child sees the father fortnightly, and the mother reports that the boy "is fine with his father and impossible with me". The matrix question is: whose unit of focus does this fit? The structural lens would look at the mother–son subsystem; the matrix reminds you that the child's split presentation across two households is a different unit, and that a model with a place for triangulation across households (or a systemic model that treats the two households as one system) may be indicated. The matrix has not told you what to do; it has stopped you from doing the obvious thing.

Case 2 — the family that has already had two therapies. A couple arrives having had behavioural couple therapy (which they found mechanical) and then six sessions of a psychodynamic approach (which they found slow). The matrix question is: what theory of change have they already tested and rejected, and what remains? A therapist who knows the grid can offer a third mechanism deliberately — an emotion-focused or narrative approach — rather than repeating what has failed, and can say why: "You have tried changing what you do and you have tried understanding where it came from. There is a third route: changing what happens between you in the moment."

Case 3 — the model you cannot deliver. A family presents with an adolescent's anorexia. You are trained in structural and narrative work and not in family-based treatment. The matrix's evidence column says the specific evidence is strongest for a model you do not practise. The professional response is visible in the grid: refer, or co-work, or seek training — and say so to the family. The matrix is at its most useful when it tells you that you are not the right clinician.

What all three cases share. The matrix is not a decision procedure that produces an answer. It is a discipline that widens the options visible to a therapist under pressure, which is where most clinical errors are made.

1.7 The reduction test and the charity test

Two habits of thought that prevent most of the errors in this course.

The reduction test. When you are tempted to say that a model is "just" something — "structural therapy is just common sense about who is in charge", "narrative therapy is just reframing with better marketing" — apply the reduction test: does the reduction preserve the model's distinctive claim about what maintains the problem? If not, the reduction is a dismissal in disguise. Structural therapy's claim is not that hierarchy is obvious; it is that hierarchy is enacted in the room, in the present, in observable space, and that changing the enactment changes the family. That claim is not common sense, and it can be tested.

The charity test. When you are tempted to defend a model against a criticism, apply the charity test: state the criticism in its strongest form, in the critic's own terms, before answering it. A model defended against a weak version of the objection has not been defended. This is the discipline that the essay assessment rewards, and it is the discipline that distinguishes a professional from a partisan.

How the two tests work together. The reduction test stops you dismissing a model too quickly; the charity test stops you defending one too quickly. Between them they produce the posture this course is trying to teach: interested, informed, uncommitted to a conclusion, and able to act.

> **PRACTICAL**
> Take one criticism of each of the three models in 1.6 — for example, "structural therapy is manipulative", "Bowen theory is untestable", "solution-focused therapy ignores suffering". State each in its strongest form in two sentences, then write the best available answer in two sentences. Bring all six sentences to the seminar.

1.8 The same ten minutes, three times

A transcript exercise that makes the grid concrete. The family is the Adeyemis; the first ten minutes are written three times, in the voice of three models, and the differences are marked.

Version A — structural.

> **THERAPIST:**
> Before we start, I want to know who is in charge of the school question in this house, because somebody has to be. Mrs Adeyemi, when the school telephones, who decides what happens? **Mother:** I do. He is not here. **Therapist:** And when he is here, at the weekend? **Mother:** Then he decides, and then he tells me I have done it wrong. **Therapist:** So in this house there are two governments, and Amara lives in both of them. (to Amara) Which one do you go to first? **Amara:** My mum. She says yes. **Therapist:** (to the parents) I want the two of you to sit together for the next ten minutes and decide one thing about Monday, while Amara and I listen. Not negotiate — decide.

What this version does. It establishes the hierarchy question in the first ninety seconds, names the coalition, addresses the child directly with a question that reveals the coalition, and moves immediately to an enactment. It uses space and action. Note what is absent: no history, no feelings, no exploration of what the family has tried.

Version B — Bowen.

> **THERAPIST:**
> I want to start with each of you, one at a time, and I want to ask about the anxiety in this house rather than about the school. Mrs Adeyemi, when Amara says she is not going, what happens inside you? **Mother:** I panic. My chest. Then I am angry. **Therapist:** And when you panic, what do you do? **Mother:** I telephone the school. I fix it. I have always fixed things. **Therapist:** You have always fixed things. Who taught you that? **Mother:** (pause) Nobody taught me. I had to. **Therapist:** And Mr Adeyemi, when you come home on Friday and hear about the week, what happens inside you? **Father:** I am tired. I say what she should have done. **Therapist:** And what happens then? **Father:** She says I am never here. Then we do not talk.

What this version does. It slows everything down, asks about inner process rather than behaviour, links the mother's reactivity to her history without interpreting it, and brings the father in as a person with a process rather than as an absent authority. It gives no task in ten minutes and does not intend to.

Version C — solution-focused.

> **THERAPIST:**
> I want to ask a different kind of question before we talk about the problem. Suppose tonight, while you are asleep, the school situation is sorted. What is the first thing you notice tomorrow morning that tells you something has changed? **Mother:** (pause) I would wake up and not feel sick. **Therapist:** You would wake up and not feel sick. What would you do then, that you do not do now? **Mother:** I would make breakfast. I would call her once, normally, and then I would sit down. **Therapist:** Amara, what would your mother see you doing? **Amara:** I would get up. **Therapist:** You would get up. When was the last morning that happened, even a bit? **Amara:** September. When my aunty's wedding was on. **Therapist:** What was different about that morning?

What this version does. It begins with a preferred future rather than a problem, produces a behavioural description, recruits the adolescent, and finds an exception inside three minutes. Note that it does not deny the problem: the mother's "not feel sick" is a precise description of suffering, elicited without a single question about the problem.

What the comparison teaches. All three versions are recognisably competent. They differ in what they treat as data, in what they do with the first ten minutes, and in what they leave for later. A student who can hear which model is speaking, from the transcript alone, has reached level 1 and is on the way to level 2.

The exercise. Read the three versions aloud in groups, one student per version, without announcing which model. The listeners identify the model and name the two sentences that gave it away. The sentences are almost always the same: the structural version gives itself away with a decision; the Bowen version with a question about inner process; the solution-focused version with a question about a future.

1.9 Laboratory: building the matrix

The week's practical runs in class and continues for six weeks. It is worth 20% of the course, and it is the single most useful document a student will produce this term.

The specification. A spreadsheet or table with nine rows (the nine schools in this course) and eight columns (the six required dimensions plus cultural assumptions and blind spot). One to three sentences per cell. Every cell must be in the student's own words: copied text is not accepted, and the plagiarism check will find it.

The build schedule. One row per week, completed before the seminar; the row for a model is discussed in the seminar and revised afterwards. By Week 7, nine rows are complete and the matrix is submitted with a covering note.

The covering note (500 words). Three questions: which model was hardest to fill and why; which cell did you change your mind about during the term; and what does the matrix as a whole tell you about the field that you did not know in Week 1?

Marking rubric.

Criterion	Weight	Full marks
Completeness	20%	Nine models × eight dimensions, no empty cells, no copied text
Fidelity to each model	30%	The theory of problem of each model is stated in that model's own terms, not reduced to another's
Derivative coherence	20%	Techniques and therapist position are visibly derived from each model's theory of problem
Evidence accuracy	15%	The evidence cell distinguishes what is strong, moderate and thin, and does not overstate
Critical columns	15%	Cultural assumptions and blind spots are specific, not generic ("all models have biases")

The two commonest failures. The first is a matrix in which every model's theory of problem is a paraphrase of the same idea — "the problem is maintained by the family's

patterns" appears nine times, which tells the marker that no model has been understood. The second is an evidence column that says "limited evidence" for every model, which is true of none of them and useless in all.

A worked cell, to show the standard. For Milan systemic therapy under theory of problem: "Problems are maintained by the family's belief system and by the punctuation of events that the family and its helpers share; the symptom is a communication in a system that has become rigid around a premise nobody has examined." That cell is specific, it is in the model's terms, and it could not be transplanted into another model's row.

1.9a An annotated matrix row, and a row that fails

Two rows for Milan systemic therapy: one that would earn full marks, one that would earn a bare pass. The comparison is the quickest way to teach the standard.

The row that works.

> Theory of problem: The family's difficulties are maintained by a belief system — a premise held by the family and shared with its helpers — and by the punctuation of events that keeps the premise unexamined; the symptom is a communication in a system that has become rigid around the premise. Unit of focus: The family's meanings, and the observing system that includes the therapy team and the referring agency. Theory of change: The premise must be made visible and disturbed; change follows from new information and difference, not from instruction. Therapist position: Curious, neutral (in the Milan sense of neutral toward all descriptions), working with a team behind a screen or, in later versions, with a reflecting team. Signature techniques: Circular questioning, positive connotation, rituals, the hypothesising-team intervention, the letter. Evidence base: Thin as a model in randomised trials; the components — circular questioning, reflecting teams, systemic hypothesising — are widely used and studied qualitatively; there is no strong specific evidence for Milan systemic therapy as a package with any presentation, and the honest cell says so.

Why it works. The theory of problem is in Milan's own vocabulary ("premise", "punctuation", "communication"), the theory of change is derived from it rather than added, the therapist position includes the team because the team is constitutive of the model, and the evidence cell distinguishes the package from the components and declines to inflate either.

The row that fails.

> Theory of problem: Family problems come from bad communication. Unit of focus: The family. Theory of change: Better communication. Therapist position: Neutral. Signature techniques: Circular questions, reframing. Evidence base: Limited.

Why it fails, cell by cell. "Bad communication" could describe five models on this course and therefore describes none. "The family" is the unit of focus for every model in the book. "Better communication" is a restatement of the problem, not a theory of change. "Neutral" is a word Milan borrowed and redefined; the row does not say what the redefinition is.

"Reframing" is not a Milan technique in the technical sense. And "limited" in the evidence cell is the flat answer that tells the marker the student has not read anything.

The lesson, stated once. A matrix row is good when a reader who knows the model could identify it from the row alone, with the model's name removed. Try that test on your own rows each week: cover the name and see whether the row is identifiable. If it could be any of three models, rewrite it.

1.10 Six errors in comparing models

1. **The catalogue.** Learning nine lists of techniques and no theories. Fix: derive the techniques from the theory of problem for every model, out loud, before you write the row.
2. **The reduction.** "They are all doing the same thing with different words." Sometimes a useful hypothesis and always an untested one. Fix: the reduction test in 1.7, applied to the model's distinctive claim.
3. **The straw model.** Comparing the strongest version of your own model with the weakest version of another's. Fix: read one primary text per model, not only the summary in a textbook.
4. **The flat evidence column.** Treating all models as equally evidenced or equally unevidenced. Fix: distinguish between specific evidence (a model tested for a presentation) and component evidence (an element used across models), and say which you mean.
5. **The cultural silence.** Filling eight columns and leaving out the assumptions that make the model possible. Fix: for each model, ask what kind of family its picture of health describes, and who that picture excludes.
6. **The premature commitment.** Choosing a favourite model in Week 2 and reading everything thereafter as support for it. Fix: the essay requires you to argue against your own preferred model; do that draft early.

1.10a The viva question bank

The viva in Week 15 examines the matrix, the case study and the position paper. These are the questions it is built from, and they can be practised in pairs from Week 1.

On comparison. "Compare two models' theories of change and say which you would choose for this family, and why." "Which two models on your matrix have the most similar theory of problem, and what follows from the similarity?" "Which cell in your matrix was hardest to fill, and what does that tell you about the model?"

On fidelity. "Demonstrate, in words, how a Milan therapist would respond to this sentence." "What would a solution-focused therapist do differently in the first five minutes, and why?" "Which sentence in this transcript is not consistent with the model, and what would the model's author have said instead?"

On evidence. "What is the strongest evidence for this model, and with which presentation?" "What would you say to a family who asked whether this approach works?" "Where is the evidence thinnest, and what do you do about that in practice?"

On critique. "What is this model's blind spot?" "What cultural assumptions does this model carry, and how would they land with this family?" "What would have to happen in a session for you to conclude that the model was the wrong choice?"

On integration. "Combine two models for this case and justify the combination, the sequence and the point at which you would switch." "What would incoherence look like in your plan, and how would you avoid it?"

On self. "Which model are you most tempted by, and what does that temptation tell you?" "What are you not yet able to do at level 2?" "What will you have changed your mind about by the end of the course?"

How to practise. Take one question per seminar, answer it in ninety seconds without notes, and let a peer ask the follow-up. The viva is twenty minutes of exactly this, and the students who do it weekly are unrecognisable by Week 12.

1.11 Culture, faith and the comparative gaze

Comparison is not culturally neutral. Three cautions.

The models were made in particular places. Structural therapy in the Philadelphia of the 1960s, Milan in the Italian psychiatric reform, solution-focused therapy in Milwaukee, narrative therapy in Adelaide and Auckland, emotion-focused therapy in Ottawa. Each carries the assumptions of its setting, including assumptions about authority, individuality, the state, the family form, and the therapist's mandate. A model exported without those assumptions examined will misread families, and will do so most confidently where the family is least like the model's origin.

The comparative gaze can become a ranking. A matrix invites the question "which is best?", and the answer will always be shaped by the culture of the assessor. A model that prizes individuation will rank a model that prizes hierarchy as backward; a model that prizes hierarchy will rank the first as naive. The professional discipline is to hold the matrix as descriptive, and to answer "best for whom, for what, in whose judgement?"

Faith enters at the level of the model's anthropology. Every model carries an implicit account of what a person is: a system of internal objects, a differentiated self, a member of a structure, a storyteller, an attachment-seeking organism. Families whose faith gives them a different anthropology — a person as a soul under authority, as a member of a covenant community — will experience some models as congruent and others as alien. The comparative question is therefore also a theological one, and it belongs in the matrix: which model's picture of a person can this family recognise?

DEBATE

"Any attempt to compare models on a single grid will favour the models that fit the grid's assumptions." Argue for and against, using one model from this course that resists the six dimensions.

1.11a Where each model's theory of problem came from

A short historical note for each of the nine schools, because a model's origin explains its blind spot more reliably than its textbook description does.

Model	Origin and founding problem	What the origin explains about the blind spot
Psychoanalytic family therapy	Post-war Britain; the problem was the internal world carried into relationships	Attends to depth; under-attends to structure, money and institutions
Bowen	Post-war America; the problem was the transmission of anxiety across generations	Attends to the multigenerational system; treats the present context thinly
Structural	Philadelphia, 1960s; the problem was the organisation of poor, multi-problem families	Attends to hierarchy and action; can miss meaning, loss and culture
Strategic / MRI	Palo Alto, 1960s; the problem was the attempted solution that maintains the symptom	Attends to sequence and to what works; can be manipulative and under-attends to consent
Milan	Italy, 1970s; the problem was the family's premises and the helpers' punctuation	Attends to belief and to the observing team; can be intellectual and slow
Experiential	America, 1960s–70s; the problem was the absence of genuine contact and feeling	Attends to immediacy and the therapist's use of self; thin on evidence and structure
Behavioural / CBT couple	America and Britain, 1970s; the problem was the reinforcement of aversive exchange	Attends to observable behaviour; can under-attend to meaning and attachment
EFT	Ottawa, 1980s; the problem was the rigid cycle organised around unmet attachment need	Attends to emotion and attachment; may under-attend to material conditions
Solution-focused	Milwaukee, 1980s; the problem was the problem-saturated description	Attends to solutions; can bypass suffering and history
Narrative	Adelaide and Auckland, 1980s–90s; the problem was the internalisation of dominant stories	Attends to meaning and power; can remain at the level of description

How to use the table. In the viva, the question "what is this model's blind spot?" is answered best by pointing to the problem the model was built to solve. Every model is a solution to a historical problem, and it inherits the blindness of that problem. A structural therapist in 1960s Philadelphia was not asked to think about colonial internalisation; a narrative therapist in 1990s Auckland was not asked to manage a psychosis relapse. The model is not at fault; the therapist who applies it outside its problem is.

1.12 Evidence note: what comparison can and cannot establish

The comparative literature is large and its findings are less exciting than students expect.

What is established. Bona fide models produce broadly similar outcomes across many presentations; differences, where they appear, are usually small and often confined to specific presentations or sub-groups. The strongest specific effects are where a model targets a well-defined mechanism in a well-defined condition — family-based treatment for adolescent anorexia is the standard example. Therapist effects are substantial, and the alliance is among the more robust predictors across models.

What is not established. That any one of the nine schools is superior overall; that model allegiance predicts outcome; that training in more models produces better outcomes. The comparative question "which model is best?" has no general answer in the literature, and clinicians who claim one are usually reporting their own experience.

What follows for the matrix. The evidence column should be filled with three questions in mind: what is the evidence for this model as a whole; what is the evidence for this model with a specific presentation; and what is the evidence for the components the model uses. A model can be weakly evidenced overall and strongly evidenced for one presentation — this is common — and a matrix that records only the first number will mislead the student who relies on it in a viva.

The professional claim. "I am using this model with this family because it targets the mechanism I believe is maintaining the problem, because there is evidence for it with this presentation, and because it is within my competence — and I will measure whether it is working." That sentence is defensible in any of the three assessments, and it is the sentence this course is designed to produce.

1.13 Key terms

Term	Definition
Theory of problem	A model's account of how difficulties develop and what maintains them
Unit of focus	What the model treats as the system of interest (individual, dyad, structure, meaning, context)
Theory of change	A model's account of what must happen, and in what order, for improvement
Therapist position	The role the model requires of the therapist, and the authority it confers
Signature technique	A characteristic method that follows from the model's theory of problem
Evidence base	The strength, quality and specificity of research support for a model
Blind spot	What a model systematically fails to notice, usually the shadow of its strength
Reduction test	Whether a proposed simplification preserves the model's distinctive claim
Charity test	Stating an objection in its strongest form before answering it
Level 1 / 2 / 3	Describe, demonstrate, critique — three separable competences with a model
Model fidelity	The degree to which a session is recognisably conducted in the model's terms
Component evidence	Evidence for an element used across models, as distinct from evidence for a whole model

1.13a Two student matrices, and what the marker sees

Two submissions, both technically complete, with very different marks. The comparison is worth an hour of seminar time.

Submission A. Nine rows, all cells filled, all in the student's own words, each model described in its own vocabulary. The evidence column distinguishes model-level from component evidence. The blind-spot column names specific consequences: "structural work with a family whose grandmother holds executive authority will read her authority as a boundary violation and will fail at the point of implementation". The cultural column is specific: "assumes a nuclear household with two parents available for enactment".

Submission B. Nine rows, all cells filled, all in generic language. "Theory of problem: dysfunctional family patterns. Unit: the family system. Theory of change: restructure the system. Position: active. Techniques: enactment, reframing, questions. Evidence: mixed." The blind-spot column says "may not suit all families". The cultural column says "must be culturally sensitive".

What the marker sees. Submission A demonstrates that nine models have been read and differentiated; submission B demonstrates that nine models have been noticed. The difference is visible in three specific places, and students should check their own drafts against them.

First, the vocabulary test. Does the row use words that belong to the model, or words that belong to the field? "Bad communication" belongs to the field; "punctuation" belongs to Milan; "the attempted solution" belongs to the strategic school; "the cycle" belongs to EFT. A matrix written in the field's vocabulary cannot distinguish the models, because the field's vocabulary is the lowest common denominator of all of them.

Second, the consequence test. Does the blind-spot column name a consequence, or a sentiment? "May not suit all families" is a sentiment. "Will fail at implementation when the family's authority structure is not the one the model assumes" is a consequence, and it tells the reader what to watch for.

Third, the honesty test. Does the evidence column record what is actually known, including the awkward parts — that Bowen theory has little trial evidence, that solution-focused therapy has better evidence for its components than for its package, that EFT's evidence is strongest for couple distress and thinner for other presentations? A matrix that is uniformly cautious is as uninformative as one that is uniformly enthusiastic.

1.14 Self-test

1. Name the six dimensions of comparison, and the two the course adds.
2. Why is the theory of problem described as the master dimension?
3. Derive the therapist position and two techniques for the Bowen model from its theory of problem.
4. What are the three levels of knowing a model, and which assessment tests each?
5. What is the reduction test, and what does it protect against?
6. What is the charity test, and why does the essay require it?

7. What three questions should the evidence column answer?
8. Give one cultural assumption carried by each of the structural and solution-focused models.

Answers. 1. Theory of problem; unit of focus; theory of change; therapist position; signature techniques; evidence base — plus cultural assumptions and the characteristic blind spot. 2. Because the account of what maintains a problem determines what the model attends to, what it does, what role the therapist takes, and what counts as change; the other five dimensions can be derived from it. 3. Position: coach, neutral observer of process. Techniques: the genogram and coaching the parents to lower their own reactivity (detriangling questions). 4. Describe (written examination), demonstrate (observed session), critique (essay and viva). 5. A test of whether a proposed simplification preserves the model's distinctive claim; it protects against dismissal disguised as summary. 6. Stating an objection in its strongest form before answering it; the essay requires it because a model defended against a weak objection has not been defended. 7. What is the evidence for the model as a whole; for this model with this presentation; and for the components the model uses. 8. Structural: that a family should have a clear hierarchy with parents in charge. Solution-focused: that a family can describe a preferred future and act on it.

1.15 Reflective task

Before the seminar, write 300 words on the following: which of the nine models in this course do you already believe, and why do you believe it? Most students discover that they hold a model without having chosen it — usually the one taught by the most persuasive lecturer, or the one that fitted their own family. Name it, state the evidence you actually have for it, and state one thing the model cannot see. Bring the paragraph; it will be revised in Week 14.

1.16 Readings for this week

- **Nichols & Davis**, chapter 1, for the survey of the field and the standard comparison frame.
- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Territory, chapters 1–2, for a systematic comparison that treats the models as answers to clinical questions.
- **Gurman & Kniskern** or **Sprenkle, Davis & Lebow (2009)**, for the common-factors argument that will challenge the matrix you are building.
- **One primary text for a model you think you already know**, read for an hour, to test whether you know it at level 1 or only at the level of the summary.
- **Carr (2019)**, the opening chapters, for the history and the structure of the field.
- **The course plan on the previous page**, read against this week's grid: for each of the nine weeks, write the one question you expect that model to ask, and bring the nine questions to the seminar. The exercise takes ten minutes and gives you a set of hypotheses to test against the reading, which is a much better use of the first week than trying to remember nine definitions you have not yet met.

Next: Week 2 — Psychoanalytic and object-relations family therapy

Week 2 — Psychoanalytic and object-relations family therapy

The question it asks. What unconscious processes, carried from earlier relationships, are being re-enacted in this family now?

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why start here	The first family therapists were psychoanalysts, and their problem is still live	Name one thing in a family that only this model explains well
10–25	Object relations in plain language	Internal working models, with two examples from the case bank	Map one family's internal objects
25–40	Projection and projective identification	The mechanism, demonstrated on a transcript	Locate the projection in a recorded session
40–55	Transference and countertransference in family work	Why the therapist's feeling is data	Identify their own response to a role-play
55–70	Interpretation, used sparingly	Four rules for an interpretation that will land	Draft and test one interpretation
70–85	The Tavistock and group-analytic traditions	Work with the family as an unconscious system	Compare with the individual model
85–90	Critique and the matrix row	What the model cannot see	Fill the row

2.2 Core concepts

The model's vocabulary is old and its mechanisms are still in use across the field, including by therapists who believe they do not use them. Learn the five concepts precisely.

Object relations. The idea that a person's early relationships are internalised as working models of self and other — a "mother who cannot be relied on", a "self who must not need anyone" — and that these internal models shape how later relationships are perceived and conducted. Family work extends the idea: each member brings a set of internal objects into the household, and the family's relationships are a negotiation between them.

Projection. Disowning a feeling in oneself and locating it in another person. A father who cannot tolerate his own fearfulness experiences his daughter as fragile and protects her excessively. The mechanism is not deception; it is usually unconscious, and it is why a family's description of one member often tells you more about the describer than the described.

Projective identification. Projection plus pressure: not only is a disowned part located in the other, but the other is recruited, often subtly, to behave in accordance with it. The mother who cannot bear her own helplessness treats her son as helpless, organises his day, speaks for him, and in time he presents as helpless — and both are sincere. This is the model's central explanatory mechanism for the identified patient, and it is the mechanism most often misapplied, because it is used to explain away the family's account.

Splitting. Experiencing people as all-good or all-bad, with the two views held separately rather than integrated. In families it appears as the alignment in which one parent is idealised and the other denigrated, or in which one child is the good one and another the bad one. The splitting is maintained by the family's anxiety: a whole object is harder to hold than two halves.

Repetition compulsion. The re-enactment of earlier relational patterns in new relationships, including with the therapist and the therapy. A family that has dismissed three therapists will dismiss a fourth, and the repetition is information.

The family as a shared unconscious system. The synthesising idea of the school: the family is not a collection of individual psyches but a system with its own unconscious assumptions, shared myths, and defended-against feelings, and one member often carries the feeling the system cannot hold. This is the clearest statement of the model's theory of problem, and it is what distinguishes the school from a set of individual psychodynamic sessions conducted in the same room.

2.2a Four concepts, one family

The concepts become learnable when they are applied to a household the student can picture. The Adeyemis, read object-relations.

The internal objects each partner brought. The mother's father died when she was 19 and she raised four siblings while her own mother was unwell. Her internal model of a reliable other is a person who nonetheless cannot be leaned on, and her model of herself is a person who copes. The father's mother died when he was 16 and he left for Lagos that year. His internal model of loss is that the way to survive it is to leave, and his model of himself is a provider who is not needed at home. Neither model is a pathology; both are adaptations that made sense in an earlier circumstance.

The projection. The mother experiences her own unmanageable grief and helplessness — about her father, about her own mother, about the years of coping alone — and locates it in Amara, who is described as fragile, unable to cope, and needing to be at home. The language the mother uses about her daughter ("she cannot face it", "she is not strong enough") is language a person might use about themselves.

The projective identification. The mother's care does not merely describe fragility; it organises the morning so that Amara is never required to manage anything. Amara is spoken for, protected from the school's letters, and relieved of the consequences. In time, Amara behaves as described — staying in her room, unable to leave — and both mother and daughter are entirely sincere.

The splitting. The family's account divides the adults cleanly. The mother holds the practical and the emotional labour and is seen as strong; the father is seen as absent and uncomprehending. Each holds one half of a whole picture, and neither can see that the mother's strength is a defence and the father's absence is a grief.

The repetition. The mother is 19 again, in effect, managing a household's feeling for other people. The father is 16 again, leaving a house in which someone has been lost. The repetition is not a curse; it is the human tendency to return to the pattern that once worked, especially under stress.

What the formulation gives and what it costs. It gives a compassionate account of both parents and a mechanism for the daughter's presentation that explains why behavioural plans have failed. It costs attention to the school's timetable, the family's money, the father's employment, and the possibility that Amara's refusal is a reasonable response to something nobody has asked about. Both halves of that sentence belong in the formulation.

2.3 Therapy: the shape and the mechanism

The shape. Longer-term than most of the models in this course, less structured, and less task-oriented. Sessions are typically weekly and open-ended; the therapist's attention is on what is happening between the members and between the family and the therapist; the work moves slowly and is expected to.

The mechanism. Two elements, both contested and both useful.

First, containment. The therapist holds difficult feelings without evacuating them — that is, without immediately turning them into advice, reassurance or a task. The family that has never had a feeling held will discover, in the holding, that the feeling can be survived. The mechanism is experiential rather than interpretive, and it is the part of the model that transfers most readily to other approaches.

Second, interpretation. The therapist offers an account of the pattern, including its unconscious dimensions, in a form the family can hear. Interpretation is not insight delivered; it is a hypothesis offered tentatively, and it works when the family's response is recognition rather than explanation.

Transference and countertransference. The family's feelings towards the therapist and the therapist's towards the family are treated as data about the family's relational world. A therapist who feels useless with a family should ask what the family does with people who try to help — a question that is far more informative than the therapist's self-criticism. A therapist who feels protective of one member should ask who else in the system feels that way, and who does not.

Why this matters even to students who will not practise this way. Every model in this course uses the therapist's response as information. This school articulated the principle first, and it is the reason supervision in family therapy attends so closely to the therapist's feelings.

2.3a How long is this therapy, and who can have it?

The model's length is not incidental; it is part of its theory of change, and a student who cannot discuss it cannot defend the matrix row.

The classical claim. Change of the kind this model seeks — a shift in internal models, in what the family can bear to feel, in the therapist's and family's capacity to stay with difficulty — is slow. The school's own logic implies months rather than weeks, and its founders generally wrote about treatment lasting a year or more.

What the modern practitioner does with that claim. Three options, each defensible.

Option 1 — practise it as designed, in a service that can fund it. Specialist psychotherapy services exist and are justifiable. The professional obligation is to be honest about the cost and the access, and to measure outcomes rather than assume them.

Option 2 — use the model's mechanisms in a shorter therapy. Containment, the therapist's response as data, the observation of what is not said, and one well-timed interpretation can all be used within a twelve-session contract. This is what most graduates will actually do, and it is not a betrayal of the model; it is the model's components deployed without its frame.

Option 3 — use it as a second phase. Some families need a structural or behavioural first phase (see MFT 701 Week 13) and can use a psychodynamic phase afterwards, when the presenting crisis is settled and the family has the resources for slower work. This is the commonest integration in practice, and it is worth being able to describe.

What none of the three options permits. Practising a slow, open-ended therapy with a family who cannot afford it, cannot reach the clinic, or needs a crisis resolved this week, and calling the mismatch "resistance". The model's silence about access is a limitation, not a licence.

The question for the viva. "You have twelve sessions with this family. What would you keep from this model and what would you drop?" A strong answer keeps the mechanisms and drops the length, names the risk (that the family experiences a short therapy as another person who did not stay), and says how it would be measured.

2.4 Signature technique: interpretation, used sparingly

An interpretation is a statement that links present behaviour to an unconscious process, offered tentatively, in the family's language, at a moment when it can be heard. Four rules separate an interpretation from an accusation.

Rule 1 — link, do not label. "When your mother talks about her own mother, you answer for her" is a link between an event and a behaviour. "You are enmeshed" is a label. Links can be tested; labels have to be accepted or refused.

Rule 2 — offer, do not pronounce. "I wonder whether..." and "I notice... could it be that..." are the grammar of interpretation. A statement delivered as fact invites compliance or defiance, and the model's own theory says the family's response to a pronouncement is also the family's work.

Rule 3 — interpret the pattern, not the person. "Something seems to happen in this room when your father begins to speak about his own father" is a statement about the system. "You are protective of your father" is a statement about a person, and it recruits the person's defence.

Rule 4 — interpret one step beyond what the family already knows. An interpretation the family has already made is tedious; one that is two steps away is a lecture. The therapist's judgement is about the size of the step, and the family's response is the evidence.

The worked example from the course material. "I notice that whenever your mother speaks about her own mother, you begin to speak for her. I wonder whether that is how the family has always managed her sadness." The interpretation has four parts: an observation (the sequence), a link (speaking for her), a hypothesis (the family's way of managing a feeling), and a tentative frame ("I wonder"). The son's answer — whether "yes, someone has to" or "no, she loses her words" — will be more informative than the interpretation itself.

What to do when the interpretation is rejected. Say "that may be wrong", and stay curious. A rejected interpretation is not a failure; it is data about the family's defences and about the therapist's timing. The model's own logic requires the therapist to survive the rejection without withdrawing, because the survival is itself the therapeutic act.

2.5 Contribution, and what the field kept

The school's contribution is larger than its current evidence base, and students should be able to say what has been absorbed into the wider field.

Kept: the respect for what is not said. Every model now attends to what a family cannot speak about. The psychoanalytic school made this a principle rather than an accident.

Kept: the therapist's emotional response as data. Now standard across models, and traceable to this one.

Kept: the idea that a symptom can carry a feeling the family cannot hold. This is the ancestor of the Milan positive connotation, the structural identification of the identified patient, and the systemic idea of function.

Kept: the family as a shared unconscious system. The notion of a family myth, a family secret and a shared assumption is now ordinary across the field, and it began here.

Kept: containment as a mechanism. Whatever the model, the therapist who can stay with a family's distress without converting it into a task is doing something this school described first.

What the field discarded, and why. Long open-ended treatment as a default; the assumption that insight is necessary for change; the pathologising vocabulary ("the intrusive mother", "the schizophrenogenic mother"); the indifference to material context; and the absence of outcome measurement. Each discard is a lesson in what happens to a model that does not examine its own assumptions.

2.5a The Tavistock tradition and group-analytic family therapy

Two institutional traditions shaped this school, and a student who cannot place them cannot read the literature.

The Tavistock. The Tavistock Clinic and the Tavistock Institute in London produced both the group-relations tradition and a substantial body of family therapy. The distinctive features are the attention to unconscious group process, the use of a work-group and basic-assumption frame, and — clinically — an interest in the family's shared assumptions and in the therapist's use of their own reaction. Tavistock-trained family therapists typically work in long-term, once-weekly formats, often with co-therapists, and often with an observing team.

Group-analytic family therapy. Derived from Foulkes's group analysis, this approach treats the family as a small group with a group matrix — a shared field of communication and meaning — and works with the group's dynamics as much as with individual members. The therapist's task is to facilitate the group's own capacity to think, rather than to interpret individual behaviour. The vocabulary is different from the classical school and the practice is often shorter and more active.

What the two traditions contribute to the matrix cell. Both extend the unit of focus: not the individual internal world alone, but the family's shared unconscious field. Both make the therapist's membership of the system explicit. Both raise the question of the observing team, which Milan later developed into a technique.

What they share with the classical school. The assumption that what is not said is the most important material; the attention to the therapist's emotional response; and a long time frame.

What a student should take from this section. Two or three named traditions, one distinguishing feature of each, and one sentence on how they differ from the classical model. That is sufficient for level 1, and it is more than most candidates can offer.

2.6 Blind spots, stated precisely

Blind spot 1 — the vocabulary blames parents. The classical language locates pathology in maternal behaviour, and the history of the school includes theories that caused real harm to mothers of children with psychosis. The modern practitioner can retain the mechanisms and must discard the blaming grammar; the test is whether the family experiences the interpretation as an accusation.

Blind spot 2 — thin evidence in its classical form. There is little randomised evidence for long-term psychoanalytic family therapy as a package. There is better evidence for psychodynamic psychotherapy in individual form for some presentations, and there is evidence for components — mentalisation, reflective functioning, containment — that the school anticipated. The matrix cell should record this distinction.

Blind spot 3 — cost, time and access. An open-ended weekly therapy is unavailable to most families in most services. A model that requires conditions most families cannot have is not wrong; it is limited, and the limit belongs in the matrix.

Blind spot 4 — the material and the institutional. A formulation that locates the difficulty in the family's internal world can miss a landlord, an employer, an immigration status or a school. The model's unit of focus is the internal and the relational; the external is under-described.

Blind spot 5 — the therapist's authority is under-examined. The interpretive position is a powerful one, and the model has historically been less attentive than the Milan or narrative schools to the power the interpreter holds.

> **CASE**
> A young man cannot keep a job; his mother calls his employers before he does. Object-relations reading: the mother's own abandonment anxiety is projected into her son's incompetence, and his failure keeps her needed. Interpretation may be less useful than an experiment: agree that she will not call, and predict that both will feel anxious for two weeks. The prediction turns insight into something testable.

2.6a What the school got wrong, in its own history

A professional course owes students the history of a model's errors, because the errors are the clearest guide to where a mechanism can be misapplied.

The schizophrenogenic mother. From the 1940s to the 1970s, a body of theory held that schizophrenia in a young person was caused by a particular kind of mother — cold, domineering, covertly rejecting. The idea was influential, was taught in good faith, and caused enormous harm: mothers were blamed for a condition now understood as neurodevelopmental, families were divided by the accusation, and a generation of clinicians was trained to see maternal care as the cause of serious mental illness.

Why the idea was plausible. It fitted the model's mechanism (the family as an unconscious system; the symptom as the container of an unheld feeling), it was supported by clinical observation of distressed families, and it explained the observations that others could not — that these families often did look unusual. What it lacked was any consideration of direction of causation: families in which a young person is psychotic look unusual because they are living with psychosis.

What the field learned, and what it cost to learn it. Three lessons, all now standard.

First, observation of a distressed family cannot establish causation. The family's state may be a response to the difficulty rather than its source. This is the single most important methodological caution in family therapy, and it applies to every model in this course, not only this one.

Second, a mechanism that explains everything explains nothing. The concept of the family as a shared unconscious system is powerful precisely because it can be applied to any family, which means it cannot discriminate between them without additional constraints.

Third, the vocabulary of a model has consequences for families. "The intrusive mother" was not a neutral description; it was a diagnosis delivered to a person who was already

struggling, and it shaped what services offered and what families believed about themselves.

How a modern practitioner uses the school despite its history. By keeping the mechanisms — projection, projective identification, containment, the therapist's response as data — while refusing the vocabulary of blame, and by asking, of every interpretation, whether the family would experience it as an accusation. That test is the professional inheritance of this history, and it should be applied to the models in Weeks 3 to 11 as rigorously as to this one.

2.7 Annotated transcript: an interpretation that lands, and one that does not

The same family, the Okonkwos, is seen by the same therapist in two sequences. The first interpretation is heard; the second is not, and the difference is instructive.

Sequence A — session three.

> **T1 THERAPIST:**
> Can I say something I have noticed, and you will tell me whether it is right? When your wife talks about Ebuka's mornings, you look at the window. When she stops, you look back. **T2 Father:** (pause) I did not know I did that. **T3 Therapist:** I wonder whether the window is where you go when the conversation is about something you cannot fix from Lagos. **T4 Father:** (long pause) It is. She tells me and I cannot do anything, so I stop hearing it. **T5 Therapist:** And then she has told you and you have not heard it, and she is alone with it. That might be the thing that is hardest in this house. **T6 Mother:** (quietly) Yes.

Annotations. T1 is an observation, offered with an explicit invitation to correct it. The observation is specific and behavioural — looking at the window — which is what allows the father to recognise it rather than dispute it. Note that the therapist does not begin with the concept.

T3 is the interpretation, and it is one step beyond what the father has said. It links the behaviour to a feeling (helplessness) and to a circumstance (Lagos), which keeps the interpretation attached to reality rather than to the father's character. The grammar is tentative.

T4 is the moment the interpretation lands, and the marker of landing is not agreement but new information: the father describes a process the therapist had only hypothesised.

T5 does the systemic work. The interpretation has been about one man's behaviour; T5 places it back in the loop, naming the consequence for the mother without blaming the father. This is the discipline that distinguishes object-relations family work from individual interpretation conducted in company.

Sequence B — the same therapist, one week later, with the same family.

> **T7 THERAPIST:**
> I have been thinking about last week, and I think I understand what is happening. Ebuka is carrying your sadness about his grandmother, because neither of you has been able to grieve, and children always carry what the parents cannot. **T8 Mother:** (silence) **T9 Father:** He is nine. He does not know anything about grieving. **T10 Therapist:** I did not mean that he understands it. I meant that he feels it. **T11 Father:** (to his wife) This is why I did not want to come.

Annotations, and what went wrong. T7 breaks three of the four rules. It is a pronouncement ("I think I understand what is happening"), not an offer; it is two steps beyond what the family knows, not one; and it interprets a person — Ebuka — rather than a pattern, in Ebuka's absence. The sentence "children always carry what the parents cannot" is a general law delivered to a family, and general laws invite families to defend themselves against the therapist's theory.

T9 is not resistance; it is a reasonable objection to an interpretation that has not been earned. The father's defence is accurate: the therapist has claimed knowledge of his son that the therapist does not have.

T10 repeats the interpretation in other words, which is the commonest response to a failed interpretation and the least useful. T11 is the cost: the father, whose engagement cost the therapist three sessions of careful work, withdraws.

The repair, done properly at the start of the next session.

> **T12 THERAPIST:**
> Last week I said something about Ebuka carrying your sadness, and it did not fit. I have been thinking about why. I think I was talking about my idea rather than about what either of you had told me. I would like to go back to what your husband said — that Ebuka is nine and does not know about grieving — because I think he was right, and I want to understand what he meant.

Why the repair works. It names the error without self-abasement, credits the person who objected, returns the authority to the family, and asks a genuine question. In this model, the therapist's survival of an error — and the family's experience of a therapist who can be corrected — is itself the therapeutic mechanism.

2.7a What the therapist brings: the use of self in a slow model

In this model more than most, the therapist's own history is an instrument, and an unexamined instrument is a hazard.

The claim. The therapist's emotional response is data (2.2), and it is also contamination if it is not examined. A therapist who was parentified as a child will feel a strong pull to protect a parentified adolescent, and the pull will arrive before the thought. The model's answer is not to eliminate the pull but to know it, which requires the therapist to have done some of the work the model asks of families.

Three disciplines, in order of importance.

First, know your own object-relations pattern. The genogram of MFT 701 and the cultural genogram are the beginning; personal therapy, where it is available and appropriate, is the standard the tradition itself assumes. A therapist who cannot name their own central relational pattern will enact it with families, reliably and invisibly.

Second, use supervision as the place where the countertransference is read. The countertransference log of 2.8a is the instrument; supervision is where the fourth column is interrogated. A supervisor's most useful question is often "and whose feeling do you think that was?"

Third, notice the families you dread. The family you avoid booking, the one you feel relief about when they cancel, the one you describe in the team meeting with a sigh — each is information about your own pattern, and each is a training opportunity rather than a personal failing.

The boundary of the discipline. Using the self is not the same as disclosing the self. A therapist's decision to share something personal should be a clinical decision with a rationale, not an expression of the countertransference. The model's own tradition is cautious here, and rightly: the family came for its own work.

The protection. Where a therapist's own history is activated repeatedly and strongly by a family, the professional response is supervision, a reduction in the caseload if possible, and personal therapy taken seriously as part of the cost of doing this work rather than as a personal indulgence. This model, of all the models in this course, requires the therapist to be a person who has been attended to, and the requirement is not a luxury; it is a competence.

2.8 Laboratory: finding the projection

A ninety-minute practical in three parts.

Part 1 (20 minutes) — the transcript. Students receive a four-page transcript of a first session with a family in which a 14-year-old is described by both parents as "the angry one". The transcript contains eleven instances of one mechanism: the parents describe their daughter's anger in language that describes their own unexpressed anger ("she is always simmering", "she cannot let anything go", "she explodes over nothing"). Students mark every instance and write the sentence the therapist could say.

Part 2 (40 minutes) — the role-play. Triads: two parents and a therapist. The parents are given a brief: each is carrying a disowned feeling (one shame, one anger), and neither knows the other's brief. The therapist's task is to find the projection without naming it as a projection, using observation, tentative linking and the family's own language. The observers score against the rubric.

Part 3 (30 minutes) — the debrief. Each therapist reports what they felt towards each parent, and the group maps those feelings onto the briefs. The exercise usually demonstrates the model's central claim: the therapist's countertransference arrived before the therapist's understanding.

Marking rubric.

Criterion	Weight	Full marks
Observation before interpretation	25%	At least three specific behavioural observations quoted before any interpretation
Tentative form	20%	Interpretations offered as hypotheses, with the family's correction invited
Pattern, not person	20%	Statements about sequences and the system rather than about a member's character
One-step calibration	20%	The interpretation is one step beyond the family's own account, not two
Survival of rejection	15%	The therapist stays curious after a rejection, and does not repeat the interpretation in other words

The commonest failure. Students interpret in the first ten minutes, before they have observed anything. The marker's note is always the same: an interpretation without an observation is a theory about the family, and families can tell the difference.

2.8a The countertransference log

The model's most transferable instrument is a single page, kept after every session. Four columns and a line.

Column	What to record
What I felt	The dominant feeling in the session, with the moment it began
What I wanted to do	The impulse — to reassure, to rescue, to withdraw, to argue, to finish early
What I did	What actually happened, which is often different
Whose feeling it might be	A hypothesis: mine; one member's; the family's shared feeling; or a response to what I represent

The line at the bottom. "What I will ask about next time."

Why the log works. It converts an unmanageable inner experience into a hypothesis that can be tested in the next session, which is exactly the move the model requires. It takes four minutes, it is the most useful supervision material a trainee can bring, and it protects against the two commonest countertransference errors: acting on the feeling without noticing it, and dismissing the feeling as irrelevant.

An example, from the Okonkwo material. Felt: irritation, then guilt, whenever the father spoke. Wanted to do: interrupt him, and then over-include him. Did: asked the mother a question instead. Whose feeling: possibly the mother's — the irritation at a man who arrives on Friday and comments on the week; possibly mine, from a family in which the father was absent; possibly the family's shared feeling about a father who is not there. Next time: ask the father what it is like to arrive on Friday into a week he has not seen.

The discipline. The fourth column must offer more than one hypothesis. A log in which every feeling is the family's is not a log; it is a way of avoiding one's own history, and supervision will find it out.

2.9 Six errors in object-relations family work

1. **The interpretation without an observation.** A theory about the family delivered before anything has been noticed. Fix: three specific observations, quoted, before any interpretation.
2. **The labelling vocabulary.** "Enmeshed", "intrusive", "over-identified" used as descriptions of a person. Fix: describe the sequence, and let the family supply the adjective.
3. **The interpretation two steps ahead.** Naming a mechanism the family has given no evidence of needing to hear. Fix: one step, and check the response before taking the next.
4. **Interpreting an absent member.** Explaining a child to their parents in the child's absence. Fix: if the person is not in the room, speak about what the parents notice, and invite the person into the next session.
5. **The failed interpretation repeated in other words.** The commonest response to rejection and the least useful. Fix: say "that may be wrong", stay curious, and return to the observation.
6. **The endless therapy.** Continuation because ending has not been planned, justified by the depth of the work. Fix: a stated review point, a theory of what would count as enough, and the discipline to say so to the family before they have to ask.

2.9a Two sessions with the same family, one object-relations and one not

A comparison that shows what the model adds, using the Ogundipe couple from MFT 701's case bank.

The family. Married fourteen years, two children. The wife says the husband is "never really there"; the husband says the wife is "never satisfied". Twice in fourteen years there has been a pushing incident, both when he had been drinking. She has not told her family. He is affectionate with the children and dismissive of his wife's account.

A structural reading. The couple subsystem is weak, the wife is in a coalition with the children, the husband is peripheral, and the wife's escalating demands are an attempt to bring him into the marriage. The first move is to strengthen the couple boundary with a protected weekly conversation and a task that requires coordination.

An object-relations reading. The wife's internal model is of a person who must manage alone, learned in a family in which her mother was depressed and her father absent; she experiences her husband's withdrawal as confirmation that she is unlovable when she needs something, and she escalates in order to extract evidence to the contrary. The husband's internal model is of a person whose needs are dangerous, learned in a family where his own mother's neediness was punished; he experiences his wife's demands as an attack and withdraws. Each is confirmed in the other's least available state. The pushing incidents are not explained by this formulation and must be handled separately and first.

What the object-relations reading adds. Three things. It explains why the escalation is so intense relative to the presenting complaint — the wife is not arguing about the school

run, she is arguing about whether she is lovable. It identifies the mechanism by which each partner's behaviour confirms the other's model, which is more explanatory than "the couple subsystem is weak". And it gives the therapist a specific countertransference prediction: the therapist will feel, towards this couple, an impulse to side with whoever seems more abandoned, and will need to notice it.

What it risks. Both formulations are about the couple's internal worlds, and neither mentions the alcohol, the pushing, or the wife's concealment from her family — which are facts about safety and about power, not about internal objects. The safety work comes first in either case, and the object-relations reading must not be allowed to soften it.

The exercise. Write the first three sentences of a session in each of the two models, then write the sentence that would open the session if the therapist had read both. Most students' third sentence is better than either of the first two, which is the case for comparison in one paragraph.

2.10 Culture, faith and the unconscious

The model's cultural assumptions are specific and consequential.

It assumes that the individual internal world is the primary unit. Many families understand a difficulty as arising between people, or between the family and the spiritual world, rather than inside a person. An interpretation offered in the vocabulary of internal objects may be heard as an accusation of individual pathology, and a family whose account is relational or spiritual will find the model's grammar foreign.

It assumes that the therapist's authority to interpret is welcome. In families where the therapist is young, of a different gender or ethnicity, or in a context where professional authority is contested, the interpretive position must be earned rather than assumed. The model's silence on this is a limitation, not permission.

It assumes a particular kind of family boundary. The concepts of enmeshment and intrusion carry a picture of the healthy family as separate individuals in a clear boundary. Families in which three generations share a household, or in which a grandmother holds executive authority, may be described in the classical vocabulary as pathological when they are simply structured differently.

Care with the spiritual register. A family may experience a difficulty as an attack, a punishment or a test, and may understand the "unconscious" as the spiritual realm. The mechanism of projection can be offered in the family's own register: "it is as though the worry has been placed in him, and now he carries it for the house." That sentence is the model's insight without the model's vocabulary, and it is more likely to be heard.

What the model offers families whose faith is central. A respect for what cannot be said, and for the reality of the unseen. A therapist who can sit with a family's spiritual account without reducing it to psychology is doing something the model requires, even if the school's founders would have described it differently.

> **DEBATE**
> "A theory that locates the problem in the parents' unconscious is a theory that blames parents." Argue for and against, using one case in which the interpretation helped and one in which it harmed.

2.10a Working with a family whose culture does not use the model's vocabulary

An exercise in translation, which is what cross-cultural practice with this model actually requires.

The family's account. A mother describes her son as "carrying something from his father's house", and says the boy "has been like this since the burial". She does not use the word grief, and she does not describe an internal world. She describes a transmissible substance — something carried, something from a house, something that arrived at a date.

The model's account. A disowned feeling has been located in the child and he has been recruited to carry it; the family's shared unconscious has organised around an unmourned loss.

The translation. "It sounds as though something came into this house at the burial and it has been in him since. I want to understand what it is, and who else in the family is carrying it — because I think more than one person is." The sentence preserves the family's own account, uses their vocabulary, and adds the model's central move (that a feeling is distributed in the system and carried by one member) without importing the model's words.

What the therapist must not do. Do not correct the family's account ("what you are describing is grief"), do not adopt it literally and abandon the clinical task ("the burial must have been done improperly"), and do not use the translation as a technique, which families detect. The translation is a way of taking the family's account seriously enough to work inside it.

The follow-up questions, in the family's register. "When he carries it, what happens in the house?" "Who else has carried something like this?" "What would have to happen for it to be put down?" Each question is the model's mechanism asked in the family's language, and each will produce material that a question about "feelings" would not have produced.

The general principle for this week. This model's power is in its attention to what cannot be said. Its danger is in its vocabulary. The therapist who can use the mechanism and speak the family's language has the best of it; the therapist who requires the family to adopt the model's language has mistaken a tool for a creed.

2.11 Evidence note: what is actually known

The matrix cell for this model must be honest, and honesty here means separating three different claims.

Claim 1 — long-term psychoanalytic family therapy as a package. There is very little randomised evidence, and no body of trials comparable to the couple-therapy literature. The evidence that exists is largely case-based, qualitative and drawn from specialist clinics with selected families. The honest cell says: "no adequate trial evidence for the package as delivered classically".

Claim 2 — psychodynamic psychotherapy in individual form. Here the evidence is better: trials and meta-analyses support psychodynamic psychotherapy for depression, some anxiety presentations and personality difficulties, with effects that appear durable after treatment ends. This is evidence for a relative of the model, not for the model.

Claim 3 — the components. Mentalisation-based treatment has trial evidence in borderline personality disorder; reflective functioning is a measured construct with predictive validity; the concept of containment has informed therapeutic work in psychosis. Attachment theory, which grew in adjacent soil, is among the best-evidenced frameworks in the field. A student who can distinguish these three claims can defend the row; a student who says "psychoanalytic family therapy is not evidenced" has said something true and uninformative.

How to use the model responsibly. As a source of mechanisms — containment, the therapist's response as data, the idea that a symptom can carry an unspoken feeling — rather than as a whole treatment to be delivered. Used this way, it is compatible with the evidence-based models in this course, and it is why the school is taught in Week 2 rather than abandoned.

2.12a A reading exercise: three paragraphs of Bion

The model's most difficult concept is containment, and the difficulty is worth meeting directly, because a student who can explain it plainly can explain anything in this course.

The passage. Take two pages of Bion — the account of the infant's fear projected into the mother, who receives it, modifies it, and returns it in bearable form. Read it three times: once for the sense, once for the words you do not know, once for what it would mean in a session.

The plain translation, written in five sentences.

> A person who cannot bear a feeling does not simply hide it; they put it into somebody else. If the other person can take it without being destroyed by it — can feel it, think about it, and give it back in a smaller form — the first person gradually learns that feelings can be survived. If the other person cannot, the feeling comes back unmodified, or the other person becomes ill with it. The therapist's job, in this model, is to be the second kind of person. That is why the therapist's own support matters: you cannot hold a feeling you have no room for.

What the five sentences leave out, and must. The developmental theory, the metapsychology, and the claim that this process is the origin of thought. Those are Bion's; the five sentences are what a family session needs.

The exercise. Read the passage, write your own five sentences, and then test them against a session: find one moment in your recorded work where a family member put a feeling into you, and describe what you did with it. Most students discover that they evacuated it — into advice, into reassurance, into a change of subject — within ninety seconds. That discovery is the most useful thing this week can produce, because it is the beginning of a skill that no model owns and every model needs.

2.12 Key terms

Term	Definition
Object relations	Internalised working models of self and other formed in early relationships
Projection	Disowning a feeling and locating it in another person
Projective identification	Projection plus pressure, recruiting the other to behave in accordance with it
Splitting	Experiencing people as wholly good or wholly bad, the views held separately
Repetition compulsion	The re-enactment of earlier relational patterns in new relationships
Containment	Holding a feeling without evacuating it into advice, reassurance or action
Transference	The family's feelings towards the therapist, read as data about their relational world
Countertransference	The therapist's feelings towards the family, read as data
Interpretation	A tentative statement linking present behaviour to an unconscious process
Working through	The repeated return to a pattern until it can be held differently
Reflective function	The capacity to understand behaviour in terms of mental states
Family myth	A shared, unquestioned belief that organises a family's account of itself

2.13a The model in three sentences, and what each sentence costs

The matrix row has to be written in the student's own words. Three sentences are enough for a good row, and each carries a cost that the blind-spot column must record.

Sentence 1 — the theory of problem. "Difficulties are maintained by internalised relationship patterns from earlier in the family's history, which are re-enacted in the present, with one member often carrying a feeling the family cannot hold." The cost: the account locates the problem in the family's internal world, which can obscure material conditions, institutions and power.

Sentence 2 — the theory of change. "Change comes through containment and through carefully timed interpretation: the family discovers that a feeling can be held rather than evacuated, and the pattern becomes thinkable rather than compulsive." The cost: the mechanism is slow, and there is little trial evidence for the package as delivered; the clinician must claim component evidence, not outcome evidence.

Sentence 3 — the therapist position. "The therapist is a participant-observer whose own emotional response is data, and whose authority to interpret must be earned in the

room." The cost: the position assumes a family able to tolerate and interested in interpretation, and it has historically under-examined its own power.

How to use the three sentences. In the matrix, in the viva, and in the first supervision session of a placement, because they are short enough to be remembered under pressure and complete enough to be challenged. A student who can say them from memory, and who can name the cost of each, has level 1 and is ready for level 2.

2.13 Self-test

1. What are the five core concepts of the school?
2. What is the difference between projection and projective identification?
3. Give the four rules for an interpretation that will land.
4. Why is the therapist's countertransference treated as data?
5. What does "containment" mean, and why is it a mechanism rather than a technique?
6. State the model's theory of problem in one sentence.
7. What are the three separate evidence claims that a good matrix cell distinguishes?
8. Name three cultural assumptions the model carries, and say what each would make hard to see in a three-generational household.

Answers. 1. Object relations; projection; projective identification; splitting; repetition compulsion (with the family as a shared unconscious system). 2. Projection locates a disowned feeling in another; projective identification also pressures the other to behave in accordance with it. 3. Link rather than label; offer rather than pronounce; interpret the pattern rather than the person; go one step beyond what the family knows. 4. Because it is information about the family's relational world, and because it is otherwise a source of enactment. 5. Holding a difficulty without converting it into advice or action; it is a mechanism because the family's experience of having a feeling survived is itself the change. 6. Difficulties are maintained by unconscious processes and internalised relationship patterns that are re-enacted in the family now, with one member often carrying a feeling the system cannot hold. 7. The package; the closely related individual modality; and the components. 8. That the individual internal world is the primary unit; that the therapist's authority to interpret is welcome; that the healthy family is one of separate individuals with clear boundaries.

2.14 Reflective task

Think of a family you know well — your own, or one from placement — in which one member is described by the others in a way that seems to say more about the describers than the described. Write 300 words on what the description does for the family, and then write the sentence you would say as a therapist, following the four rules. Bring both to the seminar. Most students find that the sentence is much harder to write than the analysis, which is the point of the week.

2.15 Readings for this week

- **Scharff & Scharff**, Object Relations Family Therapy, chapters 1–4, for the model's own account of its method.
- **Scharff & Savege Scharff (eds.)**, The Primer of Object Relations Therapy, for a shorter introduction.
- **Bion (1962)**, Learning from Experience, for containment — difficult, and worth two hours.
- **Winnicott**, any of the collected papers on the holding environment, read for the language rather than the theory.
- **Fonagy & Target**, on reflective functioning and mentalisation, for the component that has the best evidence.
- **Shedler (2010)**, "The efficacy of psychodynamic psychotherapy", American Psychologist, with the critiques that followed it, for how to read an evidence claim about this tradition — and read the critiques first, because the strongest form of the objection is the one the essay will require you to state.

Next: Week 3 — Bowen's multigenerational theory

Week 3 — Bowen's multigenerational theory

The question. How is anxiety transmitted across generations, and who in the system absorbs it?

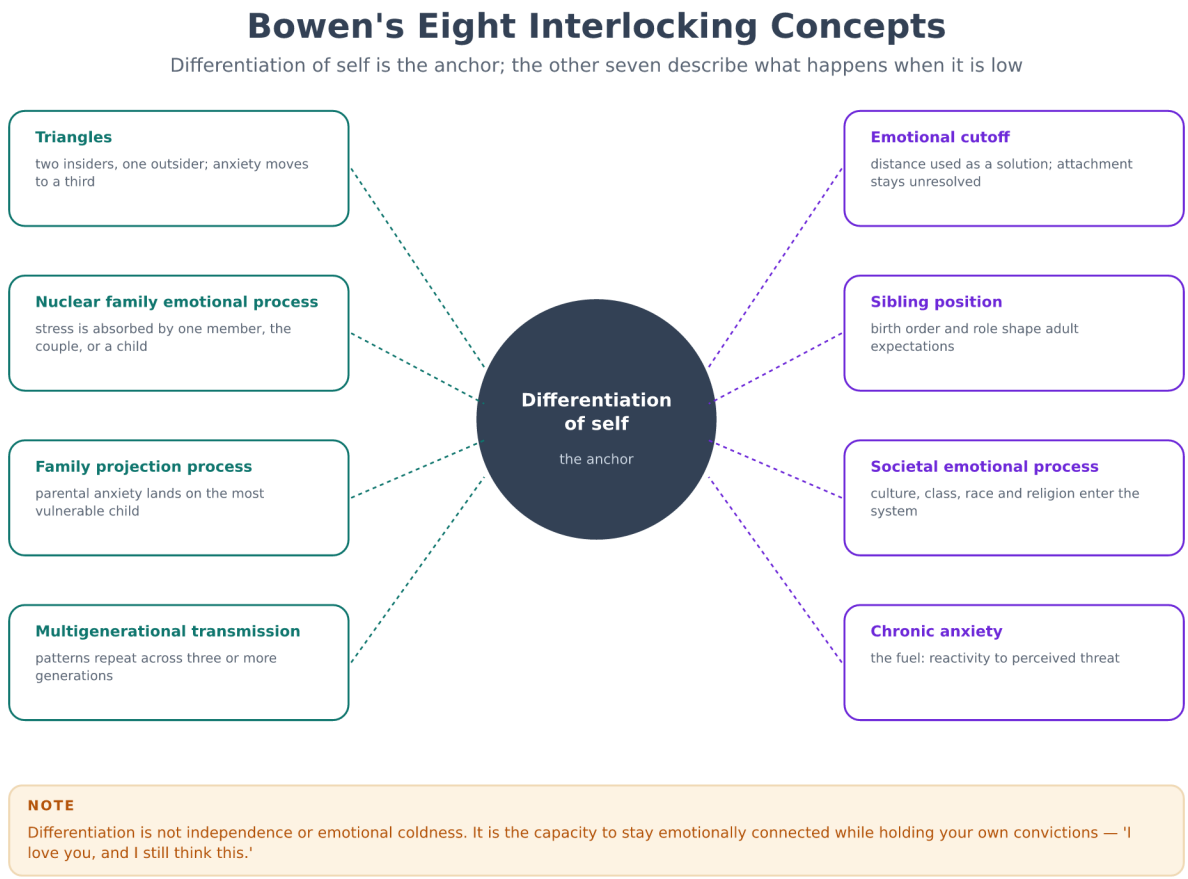


Figure 3.1 — Bowen's eight interlocking concepts

Core concepts. Differentiation of self (the capacity to stay connected while thinking for oneself); triangles; the nuclear family emotional process (marital conflict, dysfunction in a spouse, projection to a child, emotional distance); family projection process; emotional cutoff; multigenerational transmission; sibling position; societal emotional process; chronic anxiety.

Therapy. The therapist is a **coach**, not a fixer. The work is largely with the most motivated person (often "the over-functioner"), who learns to stay in contact with difficult relatives while managing their own reactivity. Genograms are the primary tool; the goal is differentiation, not symptom removal.

Signature technique — detriangling. "When your mother criticises your sister, what happens inside you? What would it look like to stay interested without taking a side and without leaving?"

Contribution. Genuinely systemic over time; explains why patterns repeat; insists the therapist do their own family-of-origin work.

Blind spots. Slow; assumes a level of choice and mobility not everyone has; can under-weight structure, gender and economics; evidence is largely clinical rather than trial-based.

> **PRACTICAL**
> Triads. One person describes a triangle in their family. The therapist practises three moves: name the triangle, ask about the client's own reactivity, and refuse to take a side. Rotate; the "client" rates how neutral (not cold) the therapist was.

Week 4 — Structural family therapy

The question. What is the organisation of this family, and how does its structure maintain the problem?

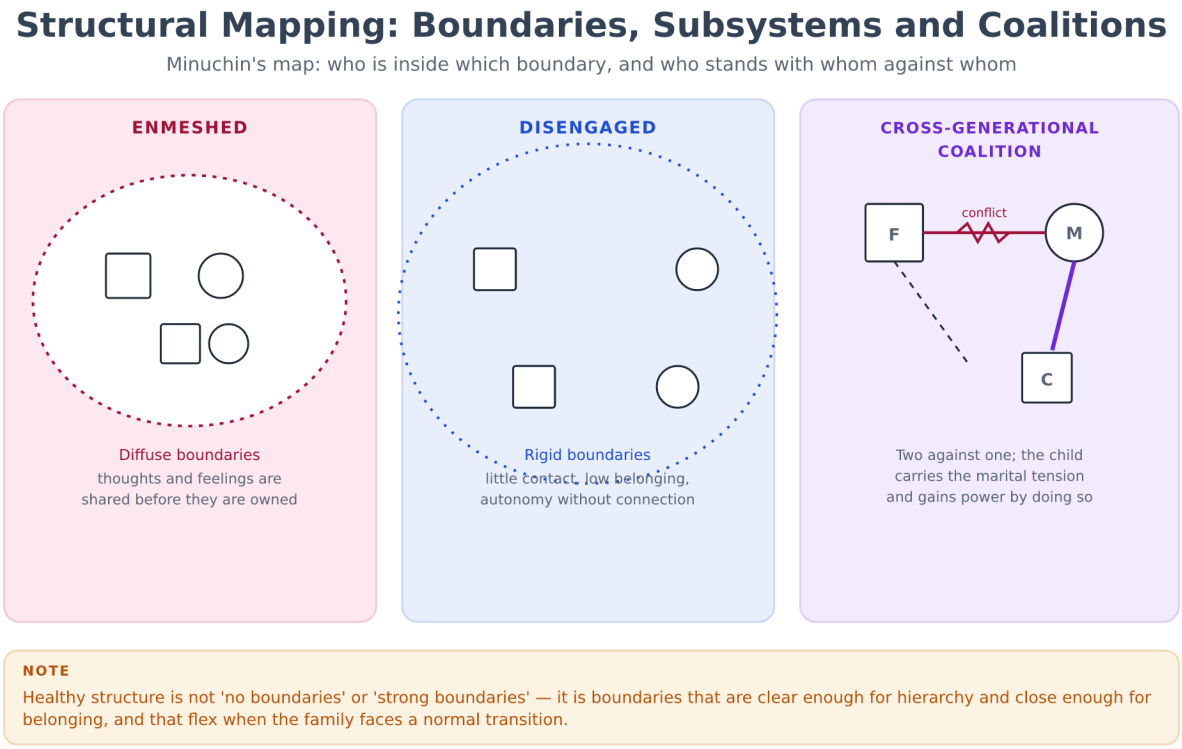


Figure 4.1 — Structural mapping: boundaries, subsystems, coalitions

Core concepts. Structure (the repetitive patterns of interaction that organise the family); subsystems; boundaries (clear, diffuse, rigid); hierarchy; alignment and coalition; enmeshment and disengagement; complementarity; the identified patient as the symptom-bearer of a structural problem.

Therapy. The therapist **joins** and then **challenges** the structure in the room. Change happens through doing, not talking: enactments, boundary-making, unbalancing, raising intensity, and marking hierarchy. Sessions are active and often directive.

Signature techniques.

- **Enactment:** "Say that to her, not to me. I'll listen."
- **Unbalancing:** deliberately supporting one member to shift the system's equilibrium.

- **Boundary-making:** "This is a conversation for the two of you; the children will wait outside."
- **Intensity:** "This is the fourth time you have laughed when your wife described the miscarriage. What is that laugh?"

Contribution. Fast, observable, highly teachable; strong evidence for structural family therapy in child and adolescent problems and in family-based treatment for substance misuse.

Blind spots. Risks imposing a middle-class norm of two-parent hierarchy; can be experienced as controlling; under-attends to inner life and to history.

> **CASE**
> The Adeyemi family. The structural reading: the parental subsystem is weak and the mother–daughter coalition crosses the generational line; the father is structurally peripheral and reacts with criticism when he is pulled in. First move: a session with the parents alone to plan one consistent rule and one shared evening, before any work with Amara. Second move: a joint session in which the parents tell Amara the plan together while the therapist blocks her attempts to negotiate with her mother alone.

Week 5 — Strategic and MRI brief therapy

The question. What is the family doing that keeps the problem going — especially their attempted solutions?

Core concepts. The problem is maintained by current interaction, not by the past; attempted solutions often persist because they "almost" work; the symptom may serve a function in the system; resistance is a response to the therapist's approach, not a property of the family; brief, focused, pragmatic.

Therapy. The therapist is an **active strategist** who designs interventions — tasks, directives, reframes, and sometimes paradoxical instructions. Sessions are few; between-session tasks do the work; termination is early by design.

Signature techniques.

- **Reframing:** "Your daughter's tantrums may be her way of keeping you two in the same room."
- **Ordeals:** a task that must be performed whenever the symptom occurs, making the symptom more costly than the change.
- **Paradoxical directive (with caution):** prescribing the symptom, under close supervision, in carefully selected cases.
- **Pretend technique:** "For one week, pretend to have the symptom on purpose for ten minutes a night."

Contribution. Broke the assumption that therapy must be long; emphasised the therapeutic power of therapist pragmatism and of clear, behavioural tasks.

Blind spots. Manipulation risks; weak evidence for paradoxical techniques; may be superficial in cases with trauma or severe mental illness; the "function of the symptom" can be speculative.

> **PRACTICAL**
> Design a one-week task for a family you know (disguised) that makes the current attempted solution impossible to continue, without instructing them to do the opposite in so many words. Present it; the group judges whether it is clear, doable, and safe.

Week 6 — Milan systemic therapy

The question. How do the family's beliefs and premises organise their behaviour — and how can curiosity disturb them?

Core concepts. The family as a meaning-making system; the **hypothesising, circularity and neutrality** stance; positive connotation (all behaviour is understood as serving the system); rituals; **circular questioning** (questions about differences and relationships rather than facts); the reflecting team and the "irreverent" later Milan style.

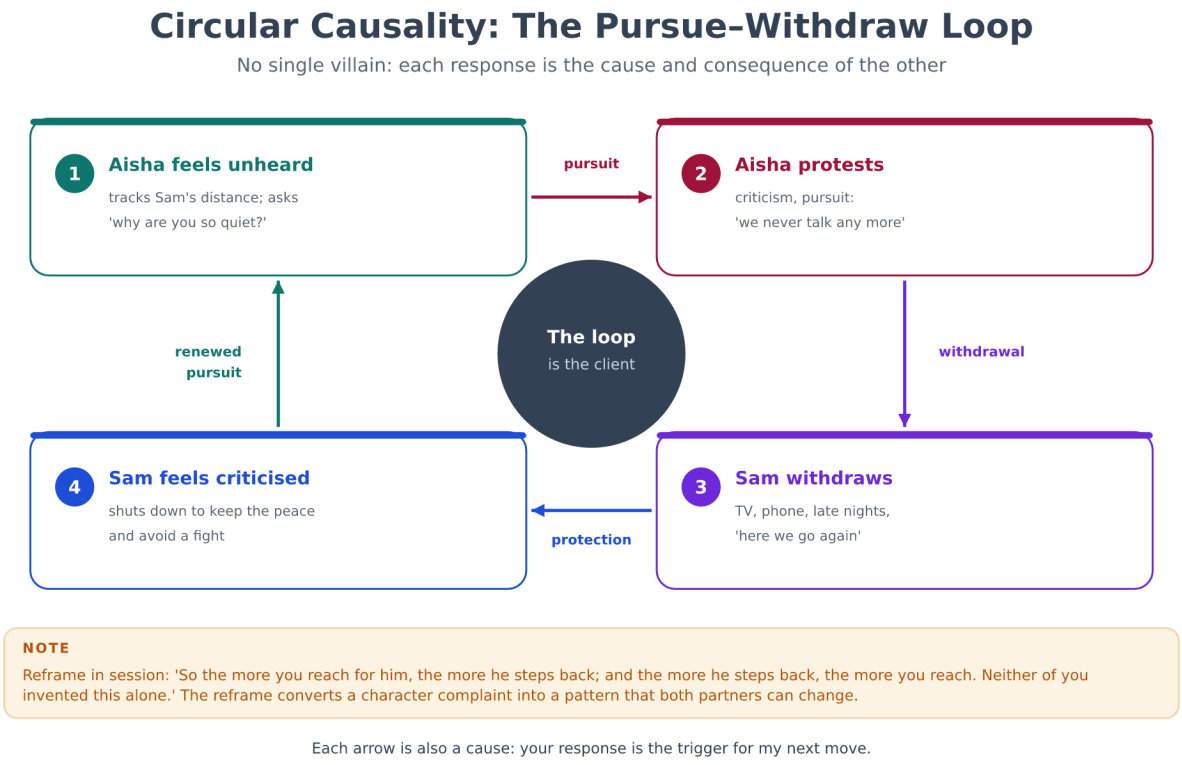


Figure 6.1 — Circular causality in practice

Therapy. The team observes behind a screen; the therapist asks circular questions; a consultation break produces an intervention that positively connotes the family's

behaviour, often prescribing a ritual. The modern version is non-directive: the team's reflections are offered as multiple perspectives, not as verdicts.

Signature technique — circular questioning.

- "Who in the family is most worried about your mother's sadness?"
- "If your brother were here, what would he say your father does when you cry?"
- "On a scale from 1 to 10, how close are your parents? And how close would they say they are?"
- "What is the difference between the way your mother treats you and the way she treats your sister?"

Contribution. Gave the field its most transferable skill — the circular question — and its most useful epistemic stance, curiosity.

Blind spots. Can be experienced as evasive in crisis; neutrality is ethically dangerous in violence; team-based format is expensive.

> **PRACTICAL**
> Ten-minute interview with a volunteer "family" (two peers). You may ask only circular questions. An observer counts how many were genuinely circular (about difference/relationship) and how many were disguised facts. Aim for 80%.

Week 7 — Experiential and human validation models

The question. What would happen if this family could feel and say the truth to each other, right now?

Key figures. Virginia Satir (family sculpture, communication stances, self-worth, "the five freedoms"); Carl Whitaker (symbolic-experiential therapy, absurdity, the therapist's use of self); Kempler and the Gestalt-influenced tradition.

Core concepts. Growth rather than pathology; the present moment as the site of change; expression of authentic feeling; the therapist's own aliveness as a therapeutic instrument; family sculpture as non-verbal enactment of structure.

Signature techniques.

- **Family sculpture:** family physically arrange themselves to show the emotional map; then rearrange.
- **Communication stances** (Satir): placating, blaming, super-reasonable, irrelevant, and congruent.
- **Here-and-now enactment:** "Tell her now. I want to see what happens to your face when you say it."

Contribution. Humanised the field; insisted on warmth, humour and the person of the therapist. Many of its insights survive inside EFT and other emotion-focused models.

Blind spots. Dependent on the charisma and health of the therapist; thin manualisation; risk of emotional flooding without containment.

> **CASE**
> In a family session, the father has not cried in 20 years. Whitaker-influenced response: the therapist does not push the tears, but stays with the father's jaw and breathing, wondering aloud about what it costs to hold it. When the father finally speaks, the therapist keeps the room still long enough for others to respond. The intervention is pace, not technique.

Week 8 — Cognitive-behavioural and behavioural couple therapy

The question. What behaviours and beliefs maintain the distress, and how can they be changed directly?

Core concepts. Behavioural exchange and communication training (TBCT); cognitive restructuring of relationship standards and attributions; the role of selective attention (noticing negatives and missing positives); homework as the engine of change; skills deficit versus skills performance.

Therapy. Structured, time-limited, with agenda and homework. Skills are taught and rehearsed in the room, then practised between sessions. Integrative behavioural couples therapy addresses both behaviour and acceptance, with attention to culture and values.

Signature techniques.

- **Behavioural exchange:** each partner lists small, achievable caring behaviours; each does them daily.
- **Communication training:** speaker–listener with a talking object (see Figure 35, MFT 716).
- **Attribution retraining:** "When he is late, what do you assume? What else could it mean?"
- **Problem-solving:** define one problem, generate options, choose, trial, review.

Evidence. Among the strongest in the field; effective for relationship distress, with good evidence for depression treated in a couple format.

Blind spots. Can feel mechanistic; may under-weight emotion and attachment; assumes a degree of motivation and literacy; "skills" can be performed in session and abandoned at home.

> **PRACTICAL**
> With a peer, design a behavioural exchange contract: three behaviours each, defined observably, cheap, daily, and mutually agreeable. Then critique it: what could make this fail in a real family within two weeks?

Week 9 — Emotionally focused therapy

The question. What attachment need is underneath the anger, and how can the cycle be de-escalated so the bond can be repaired?

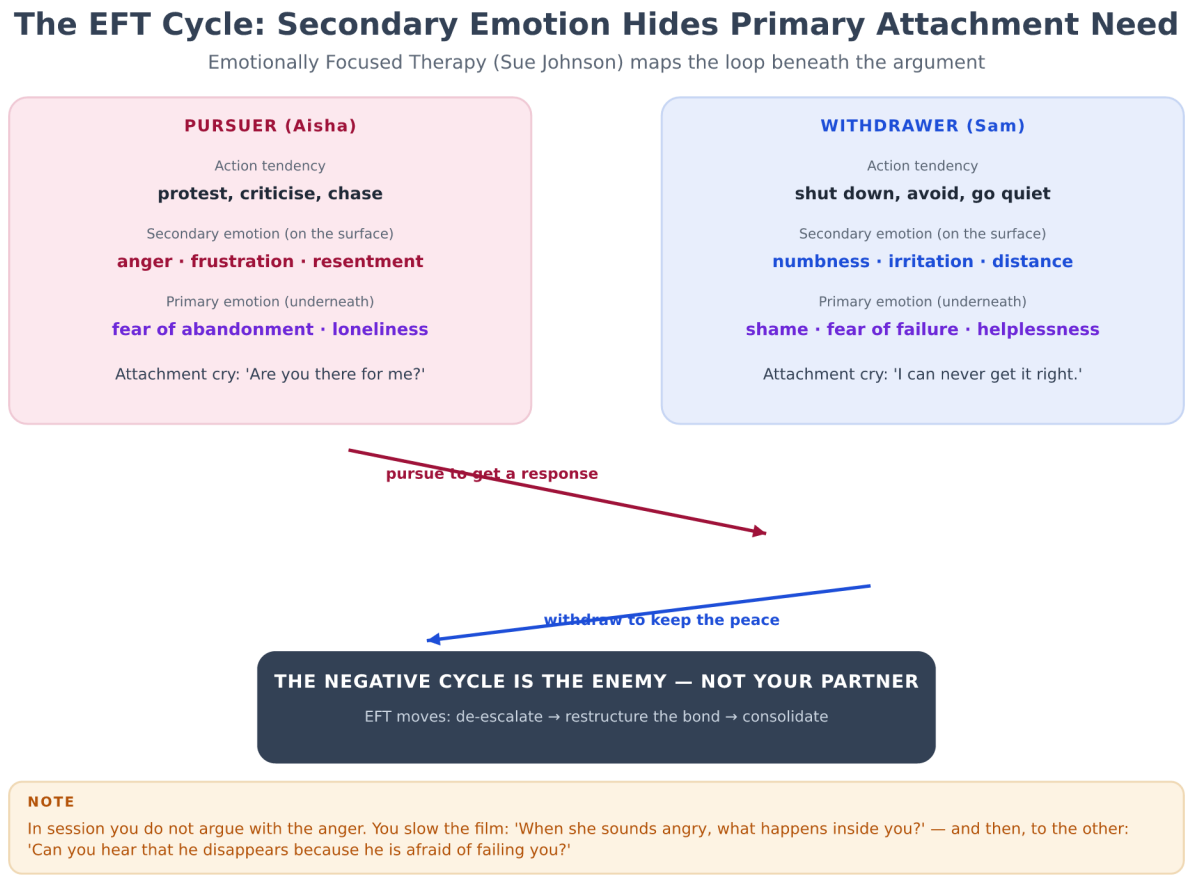


Figure 9.1 — The EFT cycle: secondary emotion and primary attachment need

Core concepts. Attachment theory as the map (see Figure 17); the negative cycle (pursue–withdraw, attack–defend, both-withdraw); primary versus secondary emotion; the "attachment cry"; softening events; the therapist as a process consultant and secure base.

Therapy in three stages. 1) **De-escalation** — identify the cycle, access primary emotion, reframe the problem as the cycle. 2) **Restructuring** — withdrawer re-engagement, pursuer softening, enactment of new responses. 3) **Consolidation** — integrate, problem-solve, prepare for recurrence.

Signature techniques.

- **Tracking the cycle:** "What happens inside you when she says that?"
- **Deepening primary emotion:** "Under the anger — is there something like fear of being alone?"
- **Enactment:** "Can you tell him that now, looking at him?"

- **Softening:** the pursuer's protest turns into an attachment expression, and the withdrawer responds.

Evidence. Among the most robust in couple therapy (over 30 trials, large effect sizes, durability in follow-up). Considered a first-line couple therapy for relational distress.

Blind spots. Requires a specific in-session emotional intensity and skills; less suited to ongoing coercive control; cultural calibration needed (the language of "needs" may feel self-indulgent to some families).

> **CASE**
> Aisha and Sam. Third session: the therapist slows a familiar argument to 20% speed. Aisha's anger is reframed as protest; Sam's silence as fear of failing. The first enactment — Aisha, quietly: "I get loud because I think you are leaving" — is the turning point. The next eight sessions are the work of making that statement safe to repeat at home.

Week 10 — Solution-focused brief therapy

The question. What is already working, and what would one small step look like?



Figure 10.1 — Solution-focused brief therapy in five moves

Core concepts. Solution talk versus problem talk; the miracle question; scaling; exceptions; coping questions; the client as expert; small, observable change; "if it works, do more of it".

Therapy. Conversational, non-normative, often 3–6 sessions. The therapist listens for competence and deliberately amplifies it; homework is usually "notice what works".

Signature techniques.

- **Miracle question:** "Suppose tonight a miracle happens and the problem is solved. What will you notice first tomorrow morning? Who else will notice?"
- **Scaling:** "On 0–10, where are you? What would one point higher look like this week?"
- **Exception questions:** "Tell me about a time the arguing stayed small. What were you doing differently?"
- **Coping question:** "With everything you have faced, how have you managed to keep going?"

Evidence. Good evidence for depression, with brief formats showing effects comparable to longer treatments in some trials; widely used in child protection and school settings.

Blind spots. Can bypass necessary grief or trauma work; risks ignoring risk; may feel invalidating when a family needs to tell the story of what happened.

> **PRACTICAL**
> Full miracle-question sequence in pairs (8 minutes), then a one-point scaling and a concrete task. Observer records: did the therapist ask for concrete behavioural detail, and did the client's posture change?

Week 11 — Narrative therapy

The question. Which story is dominating this family's life, and who wrote it?

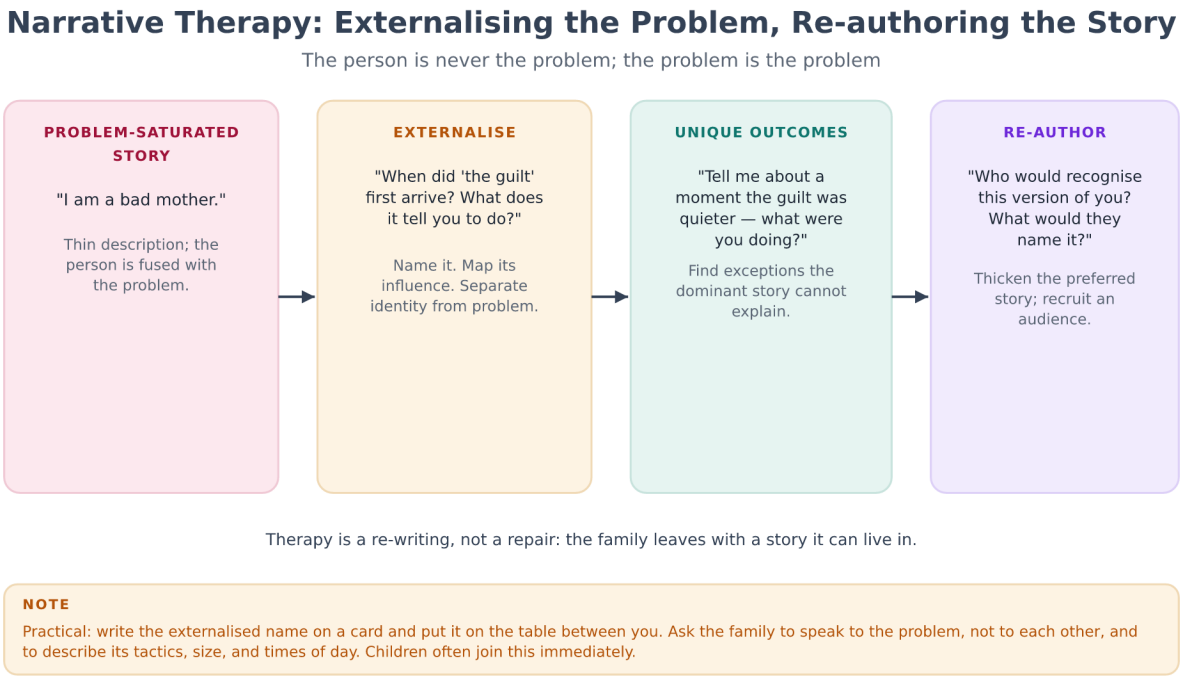


Figure 11.1 — Externalising and re-authoring the preferred story

Core concepts. The person is not the problem; the problem is the problem; dominant and preferred stories; thin versus thick descriptions; unique outcomes; externalising conversations; re-membering; definitional ceremonies; the therapist as decentred but influential.

Signature techniques.

- **Externalising:** "When did 'the guilt' first arrive in your life? What does it whisper to you?"
- **Mapping the influence:** "When does it have the most power? When does it lose power?"
- **Unique outcomes:** "Tell me about a moment when 'the shouting' was quieter."
- **Re-authoring:** "If you were to name this new version of yourself, what would you call it? Who would recognise it?"
- **Letter-writing** and **outsider-witness groups** to thicken the preferred story.

Contribution. The most culturally adaptable of the schools; central in work with children, with communities, and with people who have been defined by a diagnosis.

Blind spots. Can under-attend to material reality and risk; the linguistic sophistication can be inaccessible; evidence base is growing but smaller than EFT or CBT.

> **CASE**
> A mother referred by the school as "negligent". The therapist does not argue with the label but asks when "the exhaustion" took over, what it makes possible, what it prevents. Within thirty minutes the mother has described three occasions when she "fought the exhaustion and won", including a week when she arranged a rota with her sister. The referral, still true, is no longer the whole story.

Week 12 — Integration without incoherence

The question. How do you combine models so the client is not paying for your confusion?

Three legitimate forms of integration:

1. **Assimilative integration.** A home model, enriched by techniques from elsewhere. Most clinicians work this way. The home model keeps the formulation coherent.
2. **Sequential integration.** Different models at different phases — for example structural work to establish safety and hierarchy, then EFT for the couple's bond, then solution-focused work to consolidate.
3. **Theoretically integrated models.** Purpose-built blends (integrative behavioural couples therapy; attachment-based family therapy), coherent because they were designed together.

Rules for staying coherent.

Rule	Why
One formulation, one primary target	Prevents scattergun technique
Name the model out loud in supervision	Makes your reasoning auditable
Change model for a reason you can state	"Because the conflict escalates when we go near feelings"
Do not mix incompatible theories of change in one move	Reframing and behavioural contracting pull in different directions
Review at fixed intervals	Integration fails by drifting, not by decision

Trap to avoid: technique salad. Symptom-tracking from CBT, a miracle question, a family sculpture and an externalising conversation in one session does not make you integrative; it makes you unpredictable, and the family cannot tell what is expected of them.

> **ASSIGNMENT (30%)**
> One disguised case; two models; one formulation each; a reasoned choice of sequence; a predicted risk of the combination; a plan for how you would know it is not working.

Week 13 — Evidence, culture and critique

Model	Strongest evidence	Weakest evidence	Main cultural critique
Structural	Child/adolescent behaviour; substance misuse	Adult mood disorders	Imposes a particular family hierarchy
Bowen	Clinical tradition; modest outcome data	Few RCTs	Assumes mobility and choice; slow
Strategic/MRI	Some RCTs in child problems	Paradoxical techniques	Manipulative; may ignore trauma
Milan	Weak in classical form	Rituals, team format	Neutrality unethical in violence
Experiential	Moderate, largely process research	Manualisation	Charisma-dependent; middle-class
CB couple therapy	Strong	Severe relational trauma	Mechanistic; literacy-dependent
EFT	Strong, replicated	Coercive control	Individualist language of needs
Solution-focused	Moderate (depression, brief formats)	Severe mental illness	Can bypass grief and risk
Narrative	Growing; strong qualitative base	Large trials	Requires verbal fluency

The reflexive question. For each row, ask: whose family is this model describing? Models were developed largely with white, Western, middle-class, two-parent families in clinic settings. When you apply them to a polygynous household, a three-generation compound, a family separated by migration, or a family whose authority structure is religious rather than developmental, you must translate — and say that you are translating.

Week 14 — Your theoretical position

Write 1,500 words: your home model, why it fits you, what it cannot do, the models you borrow from and under what conditions, and how you will know when to change your mind. Defend it in a ten-minute viva.

Integration: nine models on one page

Model	Problem is…	Change happens by…	Therapist is…	First move
Object relations	Unconscious re-enactment	Making the unconscious usable in the room	Container/interpreter	Notice the transference; interpret gently
Bowen	Undifferentiated anxiety across generations	Differentiation of self	Coach	Genogram + detriangling

Model	Problem is…	Change happens by…	Therapist is…	First move
Structural	Dysfunctional organisation	Restructuring in the session	Director/joiner	Enactment; strengthen hierarchy
Strategic	Attempted solutions	Interrupting the sequence	Strategist	Task, reframe, directive
Milan	Meaning premises	Curiosity disturbing belief	Curious team member	Circular questions
Experiential	Blocked feeling and authenticity	Present-moment expression	Real person	Sculpture; here-and-now
CB couple	Behaviour and attribution	Skills and rehearsal	Teacher/coach	Behavioural exchange
EFT	Attachment injury and cycle	Accessing primary emotion; enacting new responses	Process consultant	Track and reframe the cycle
Solution-focused	Problem-saturated attention	Amplifying existing solutions	Collaborator	Miracle/scale question
Narrative	Dominant story	Externalising and re-authoring	Decentred, influential	Externalising question

Glossary

Circular questioning — questions about differences and relationships that reveal pattern rather than fact. **Differentiation of self** — the capacity to stay emotionally connected while holding one's own position. **Enactment** — directing family members to speak to each other in the session rather than through the therapist. **Externalising** — locating a problem outside a person, as a thing with tactics and a history. **Negative connotation / positive connotation** — interpreting behaviour as serving the system, rather than as defective. **Paradoxical directive** — prescribing the symptom in order to change its function (used rarely, with supervision). **Reframe** — relabelling behaviour within a new frame that makes change possible. **Restructuring** — changing the family's organisation in the session through enactment and boundary work. **Scaling** — asking a family to rate a state on a numerical scale to make change visible and specific. **Unique outcome** — a moment when the dominant story did not hold, used as a foothold for a preferred story.

Self-assessment

Multiple choice

1. "The problem is the problem; the person is not the problem" is a principle of: (a) structural (b) narrative (c) Bowen (d) strategic

2. A therapist says, "For the next week, I want you to argue for exactly fifteen minutes at 8 p.m., then stop." This is closest to: (a) narrative (b) EFT (c) strategic (d) solution-focused

3. Positive connotation is associated with: (a) Milan (b) CB couple therapy (c) object relations (d) solution-focused

4. Which model places differentiation of self at its centre? (a) structural (b) Bowen (c) EFT (d) Milan

5. "Who would notice first if the problem were solved?" is a technique from: (a) structural (b) solution-focused (c) narrative (d) experiential

6. In EFT, the behaviour that conceals a primary attachment emotion is called: (a) enactment (b) secondary emotion (c) reframe (d) restructuring

7. Which model is most associated with family sculpture? (a) Satir (b) Haley (c) Minuchin (d) White

8. "Unbalancing" is a technique of: (a) structural therapy (b) solution-focused therapy (c) object relations (d) narrative therapy

9. Which model has the strongest replicated randomised evidence in couple therapy? (a) Bowen (b) Milan (c) EFT (d) experiential

10. Assimilative integration means: (a) blending theories equally (b) using a home model enriched by borrowed techniques (c) alternating models by session (d) refusing to commit to a model

Answer key

1 b · 2 c · 3 a · 4 b · 5 b · 6 b · 7 a · 8 a · 9 c · 10 b

Short answer

1. State the theory of change of structural therapy in two sentences, then name one case type where it is a poor fit and say why.
2. Write three circular questions you would ask a family in which a father has just lost his job.
3. What is the difference between positive connotation and reframing?
4. Give two reasons why narrative therapy suits work with children.
5. Name two incompatible assumptions that would make a structural-plus-narrative formulation incoherent.

Essay (choose one, 2,000 words)

1. Compare structural and EFT approaches to the same couple case. Which would you choose first and why?
2. "Neutrality is a luxury the powerful can afford." Discuss with reference to Milan therapy and domestic violence.

3. Critically evaluate the claim that theoretical integration is a sign of maturity rather than confusion.

Further reading

Comparative

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- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
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Next: MFT 703 — Human Development Across the Lifespan